

Access guide — mcrc-kras-a59t-pik3ca-peritoneal-pre-hi pec-q8k4

How to access each therapy: trial contacts + manufacturer medical-info

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How a patient or treating team could practically access each intervention in this case's dossier. Trial recruitment contacts and manufacturer medical-information lines are captured for direct outreach. The number in the first column of the summary table is the entry's identifier in the per-intervention sections below — use it to find the deep section quickly. Information ages — each row carries a [Verified](#) date; re-screen before relying on a specific contact or trial slot.

Summary

#	Intervention	Modality	Access status	Regulatory	Recommended first action
1	Aflibercept (Zaltrap) + FOLFIRI	fusion_protein	Standard of care	FDA-approved August 2012 (VELOUR) for metastatic CRC progressing after oxaliplatin, in combination with FOLFIRI. EMA-approved with comparable wording.	Park aflibercept as a labeled second-line antiangiogenic if the patient eventually switches to FOLFOX and progresses; not in immediate scope while FOLFIRI-bev is active.
2	Bevacizumab (Avastin) and biosimilars (Mvasi, Zirabev, Vegzelma, Alymsys, Abevmy, Aybintio)	monoclonal_antibody	Standard of care	FDA-approved 2004 for first- and second-line mCRC in combination with chemo. Multiple biosimilars approved (Mvasi 2017, Zirabev 2019, Vegzelma 2022, Alymsys 2022, Aybintio 2022, Abevmy 2024). EMA-approved with comparable wording.	Coordinate the bevacizumab pause with the CRS-HIPEC team: last dose at least 4 weeks before surgery, ideally 6, and no restart until the abdominal wound is healed.
3	Capecitabine + bevacizumab maintenance (CAIRO3)	chemotherapy_plus_biologic	Standard of care	Component drugs FDA-approved. Maintenance use is published regimen, not a labeled indication; routinely covered.	Decide on a post-HIPEC maintenance plan with the medical-oncology team. If maintenance is chosen, the cape-bev backbone is the operational standard.

4	Cytoreductive surgery + HIPEC (mitomycin C or oxaliplatin)	surgical_procedure	Standard of care	Surgical procedure plus off-label intraperitoneal use of approved chemo drugs. Not a labeled drug indication; HIPEC delivery is institution-specific protocol.	Confirm the CRS-HIPEC center is high-volume (>20 cases/year) and uses mitomycin C as the default HIPEC agent; the post-PRODIGE-7 consensus is that oxaliplatin HIPEC adds toxicity without OS benefit.
5	FOLFIRI (5-FU + leucovorin + irinotecan)	chemotherapy	Standard of care	Each component FDA-approved decades ago: irinotecan (Camptosar, 1996), 5-FU (1962), leucovorin (1952). All generic; FOLFIRI is a generic combination regimen.	Confirm UGT1A1 *28 genotype and DPYD status before any dose change (target_validation has these as essential rows).
6	FOLFOX + bevacizumab (mFOLFOX6-bev)	chemotherapy_plus_biologic	Standard of care	Oxaliplatin FDA-approved 2002; 5-FU, leucovorin, bevacizumab approved; biosimilar bevacizumab options Mvasi / Zirabev approved.	Bank FOLFOX as the post-HIPEC adjuvant option since the patient is oxaliplatin-naïve — saves the cleaner second-line drug for the moment it is most needed.
7	FOLFOXIRI + bevacizumab (TRIBE regimen)	chemotherapy_plus_biologic	Standard of care + Off-label use	Component drugs all FDA-approved. The FOLFOXIRI combination is a published regimen rather than a labeled product; bevacizumab is on-label as a first-line mCRC combination partner (Avastin label).	Reassess whether triplet intensification is wanted given the patient is already tolerating FOLFIRI-bev. CAIRO6 is the protocol-shaped template even though it is closed to new enrollment.
8	FoundationOne Tracker (Foundation Medicine tumor-informed ctDNA MRD)	diagnostic_assay	Standard of care	LDT; Medicare-covered under MoIDx LCD. CLIA-certified.	Use only if FoundationOne CDx is the master NGS panel; otherwise default to Signatera.

9	Fruquintinib (Fruzaqla)	small_molecule	Standard of care	FDA-approved November 8, 2023 (FRESCO-2) for previously treated metastatic CRC after fluoropyrimidine, oxaliplatin, irinotecan, anti-VEGF, and (if RAS-WT) anti-EGFR therapy. EMA-approved June 2024.	Reserve fruquintinib as the post-FOLFOX, post-bevacizumab labeled antiangiogenic for 3L use.
10	Guardant Reveal (Guardant Health tumor-naïve ctDNA MRD)	diagnostic_assay	Standard of care	test (LDT). Medicare-covered under MoDx for CRC MRD surveillance. CLIA-certified.	Default to Signatera for the peri-HIPEC plan (target_validation prefers Signatera). Reserve Guardant Reveal as the fallback if no tissue is available.
11	Low-dose aspirin (PIK3CA-mutant rationale, ALASCCA)	small_molecule	Standard of care + Off-label use	Over-the-counter; FDA-approved for cardiovascular prevention. Anti-cancer use is off-label.	Discuss low-dose (81 mg) aspirin with the medical oncology team based on the ALASCCA PIK3CA-mutant signal.
12	Perioperative systemic chemo + CRS-HIPEC (CAIRO6 schema)	treatment_strategy	Standard of care	Strategy combining approved drugs and standard surgical procedure. CAIRO6 is the registrational trial.	Coordinate the systemic-to-surgical-to-systemic transition with the medical oncology, surgical oncology, and HIPEC teams at the chosen center.
13	Ramucirumab (Cyramza) + FOLFIRI	monoclonal_antibody	Standard of care	FDA-approved April 2015 (RAISE) for second-line mCRC in combination with FOLFIRI after bevacizumab + oxaliplatin + fluoropyrimidine. EMA-approved with similar wording.	Hold as a sequence option for 2L+ after oxaliplatin exposure. No immediate action.
14	Regorafenib (Stivarga)	small_molecule	Standard of care	FDA-approved 2012 for refractory mCRC. ReDOS dose-escalation (80→120→160 mg) is the operational standard.	Bank as a 3L+ antiangiogenic. REGONIVO (regorafenib + nivolumab) is the MSS-CRC immune-combination precedent but did not move the needle in confirmatory trials.

15	Signatera (Natera tumor-informed ctDNA MRD)	diagnostic_assay	Standard of care	test (LDT). Medicare-covered for stage II-IV CRC MRD surveillance under MoIDx LCD. CLIA-certified at Natera.	Order Signatera baseline pre-CRS-HIPEC; Natera will design the bespoke 16-variant panel from the FFPE block.
16	Tempus xM (Tempus tumor-informed ctDNA MRD)	diagnostic_assay	Standard of care	LDT; Medicare-covered for select CRC MRD indications.	Use only if Tempus xT is the master NGS panel; otherwise default to Signatera.
17	Trifluridine/tipiracil (Lonsurf, TAS-102)	small_molecule	Standard of care	FDA-approved September 2015 (RECURSE) for refractory mCRC. Label updated 2023 (SUNLIGHT) to include the trifluridine/tipiracil + bevacizumab combination.	Bank as a 3L+ option, paired with bev under the SUNLIGHT regimen.
18	Trifluridine/tipiracil + bevacizumab (SUNLIGHT)	chemotherapy_plus_biologic	Standard of care	FDA label updated August 2023 to include the trifluridine/tipiracil + bevacizumab combination (SUNLIGHT). Both components individually approved.	Bank for the 3L+ sequence after FOLFIRI-bev and FOLFOX exposure.
19	Alpelisib (Vijoice / Piqray, BYL719)	small_molecule	Off-label use + Clinical trial	FDA-approved May 2019 (Piqray, SOLAR-1) for PIK3CA-mutant HR+/HER2- advanced breast cancer in combination with fulvestrant. FDA-approved April 2022 (Vijoice) for PIK3CA-related overgrowth spectrum (PROS) in pediatric and adult patients. EMA-approved.	Default toward inavolisib (INTRINSIC, NCT04929223) for the PI3K-alpha axis; alpelisib does not have a US trial slot for mCRC.

20	Anti-EGFR mAbs (cetuximab / panitumumab) in mCRC — evidence-summary entry	monoclonal_antibody	Off-label use	Composite category; see cetuximab and panitumumab rows.	Use the cetuximab and panitumumab rows for operational decisions; this row captures the meta-evidence summary.
21	Capivasertib (Truqap, AZD5363)	small_molecule	Off-label use	FDA-approved November 2023 (CAPItello-291) for PIK3CA / AKT1 / PTEN-altered HR+/HER2-advanced breast cancer in combination with fulvestrant. EMA-approved 2024.	Do not pursue off-label outside a trial. AKT-inhibitor activity in PIK3CA-mut mCRC is mechanistically rational but clinically thin.
22	Celecoxib (Celebrex) — PIK3CA-mutant adjuvant rationale (CALGB/SWOG 80702)	small_molecule	Off-label use	FDA-approved 1998 for arthritis pain. Off-label as CRC adjuvant.	Default to aspirin (ALASCCA) rather than celecoxib for the PIK3CA-axis adjuvant signal. Celecoxib does not add over aspirin in this case.
23	Cetuximab (Erbix)	monoclonal_antibody	Off-label use	FDA-approved 2004 for KRAS wild-type, EGFR-expressing mCRC (left-sided preferred). The FDA label explicitly excludes KRAS-mutant tumors based on extended-RAS analyses (CRYSTAL, FIRE-3, CALGB/SWOG 80405). EMA-approved with similar exclusion.	Treat single-agent or chemo-combination cetuximab as off the table outside a trial; do not bet payer effort on the panitumumab case-report precedent.
24	Inavolisib (Itovebi, GDC-0077)	small_molecule	Off-label use + Clinical trial	FDA-approved October 2024 (INAVO120) for PIK3CA-mutant, HR+, HER2-negative, endocrine-resistant advanced breast cancer in combination with palbociclib and fulvestrant. EMA approval pending. No mCRC approval.	Email or call 888-662-6728 with the PIK3CA M1043I + KRAS A59T molecular profile to ask which INTRINSIC arm fits.

25	Nivolumab + ipilimumab (CheckMate 142, MSI-H mCRC)		Off-label use	FDA-approved July 2018 (CheckMate 142) for MSI-H / dMMR mCRC. CheckMate 8HW expanded the 1L MSI-H combination indication.	Skip.
26	Panitumumab (Vectibix)	monoclonal_antibody	Off-label use	FDA-approved 2006 for KRAS wild-type mCRC. Label updated 2014 to include extended-RAS wild-type. EMA-approved with similar wording.	Confirm the KRAS A59T VAF and that the rest of the extended RAS panel (KRAS exon 2/3/4, NRAS, HRAS) and BRAF V600 are wild-type before any anti-EGFR conversation.
27	Pembrolizumab (Keytruda) — MMR-deficient mCRC indication	monoclonal_antibody	Off-label use	FDA-approved May 2017 (tumor-agnostic MSI-H/dMMR); approved 2020 (KEYNOTE-177) for first-line MSI-H/dMMR mCRC. Tumor-agnostic TMB-H (≥ 10 mut/Mb) approval June 2020.	Skip as monotherapy. The ICI question routes through BOT/BAL combinations (see botensilimab-balstilimab row).
28	Regorafenib + nivolumab (REGONIVO)	small_molecule_plus_mab	Off-label use	Each drug FDA-approved separately. The combination was not approved for mCRC; the confirmatory trial (NCT04126733) did not move the needle on the Japan-cohort signal.	Skip. Route ICI to BOT/BAL combinations.
29	Trastuzumab + lapatinib (HERACLES regimen)	small_molecule_plus_mab	Off-label use	Each component FDA-approved (trastuzumab 1998; lapatinib 2007 for HER2+ breast). Combination use in mCRC is off-label; HERACLES was an Italian Phase 2 study not registrational for a CRC label.	Skip. Route HER2 question to T-DXd if HER2-amplified.

30	Trastuzumab deruxtecan (Enhertu, T-DXd)	antibody_drug_conjugate	Off-label use	FDA-approved April 2024 for HER2-positive (IHC 3+) metastatic solid tumors (tumor-agnostic). The HER2-low and HER2-ultralow breast labels are separate. mCRC use was previously off-label evidence) but now sits inside the pan-tumor IHC 3+ label.	Order the HER2 IHC + ISH per target_validation; without IHC 3+ (or ISH amp consistent with IHC 3+), this row is moot.
31	Tucatinib + trastuzumab (Tukysa + Herceptin, MOUNTAINEER)	small_molecule_plus_mab	Off-label use	FDA-approved January 2023 (MOUNTAINEER) for HER2-positive (IHC 3+ or ISH-positive), RAS wild-type metastatic CRC after fluoropyrimidine, oxaliplatin, irinotecan, and an anti-VEGF agent. EMA-approved with comparable wording.	Wait on the HER2 IHC + ISH result before doing anything (target_validation has this as essential). If HER2 is negative, this row closes.
32	AZD0466 AZD4320 dual Bcl-2 / Bcl-xL)	small_molecule_dendrimer	Clinical trial	Investigational. Phase 1 first-in-human (NCT04214093 closed; NCT04865419 active in select hematologic indications).	Email AZ medical info (1-800-236-9933) to ask whether an mCRC solid-tumor expansion is in active development. Default is to not pursue.

33	Botensilimab + balstilimab (BOT/BAL, Agenus)		Clinical trial + Expanded access program	Investigational. Phase 3 BATTMAN (NCT07152821) is the registrational mCRC trial. Agenus has an FDA-authorized Expanded Access Protocol (NCT06751524) covering multiple cancer types including CRC. France: AAC national access for ovarian and soft-tissue sarcoma.	Most actionable today: contact USC's Charlean Ketchens 323-865-0451) about NCT06336902 — the KRAS-mutant mCRC cohort is the structural match.
34	Camonsertib (RP-3500, Repare Therapeutics)	small_molecule	Clinical trial	Investigational. Repare Therapeutics ATR inhibitor; Phase 1/2 TRESR (NCT04497116) basket completed primary analysis; phase 2 expansion in selected biomarker-positive tumors.	Email clinicaltrials@reparerx.com to ask whether TP53 R273-mutant mCRC fits any active expansion cohort.
35	Ceralasertib (AZD6738)	small_molecule	Clinical trial	Investigational. AstraZeneca ATR inhibitor; multiple combination trials in melanoma, NSCLC, HCC.	Email AZ Medical Information (1-800-236-9933) to ask whether any active ceralasertib basket accepts TP53-mutant mCRC patients.
36	DT2216 (BCL-xL PROTAC degrader)	PROTAC_degrader	Clinical trial	Investigational. FDA Orphan Drug Designation 2022 and Fast Track designation 2023 for T-cell lymphoma. Phase 1 monotherapy in solid tumors completed. No regulatory approval.	Email info@dialectictx.com to ask whether an adult mCRC arm or a compassionate-use protocol is planned. Document the BCL2L1 amplification status (target_validation amplification-boundary-20q row) when reaching out.

37	Daraxonrasib (RMC-6236)	small_molecule	Clinical trial	Investigational. FDA Breakthrough Therapy designation for previously treated metastatic KRAS G12X PDAC (2024); FDA Expanded Access Protocol authorized May 2026 for previously treated metastatic PDAC only (not CRC). No FDA, EMA, or PMDA approval for mCRC.	Email medinfo@revmed.com or call 1-844-2-REVMED with the patient's KRAS A59T VAF and clinical summary; ask explicitly whether atypical codon 59 mutations are accepted into the CRC arms of RMC-GI-102 (NCT06445062).
38	ELI-002 (Elicio 7P KRAS amphiphile peptide vaccine)	vaccine	Clinical trial	Investigational. Phase 1/2 AMPLIFY-201 / AMPLIFY-7P in MRD-positive resected CRC and PDAC. No regulatory approval.	Skip. The KRAS antigen panel does not include A59T.
39	Elimusertib (BAY 1895344)	small_molecule	Clinical trial	Investigational. Bayer ATR inhibitor; Phase 1 combination trials.	Email Bayer Medical Information (medinfo@bayer.com) to ask about active elimusertib mCRC baskets.
40	GRT-C901 + GRT-R902 (Gritstone personalized neoantigen vaccine)	vaccine	Clinical trial	Investigational. Phase 2/3 GO-010 in MSS mCRC.	Skip. Route the personalized-vaccine question to KRAS peptide + BOT/BAL (Johns Hopkins NCT06411691) if vaccines remain in interest.
41	JAB-23E73 (Jacobio pan-KRAS inhibitor)	small_molecule	Clinical trial	Investigational. Phase 1/2 first-in-human. No regulatory approval.	Email or call 781-918-6670 with the patient's KRAS A59T VAF and ask which of the four sites has open screening slots.
42	KRAS peptide vaccine + Poly-ICLC + balstilimab + botensilimab (Johns Hopkins)	vaccine_plus_ici	Clinical trial	Investigational.	Email GIClinicalTrials@jhmi.edu or call Colleen Apostol RN (410-614-3644) with the patient's KRAS A59T molecular profile to ask whether the vaccine antigen set includes codon 59.
43	Pelcitoclax (APG-1252)	small_molecule	Clinical trial	Investigational. Phase 1 / 2 trials ongoing, mostly Asian sites; some US clinical-trial activity.	Email info@ascentage.com to ask about adult mCRC expansion plans; default is not to pursue today.

44	Pressurized intraperitoneal aerosol chemotherapy (PIPAC)	surgical_procedure	Clinical trial	Investigational in the US. PIPAC aerosolization equipment (Capnopen) is CE-marked in EU; not FDA-cleared as a system, so PIPAC is delivered under research protocols in the US.	Reserve PIPAC for post-HIPEC peritoneal recurrence; not in scope while the upfront CRS-HIPEC is the plan.
45	S241656 (Servier ERK inhibitor, BDTX-4933-101)	small_molecule	Clinical trial	Investigational. Phase 1/2 first-in-human. No regulatory approval.	Email with the patient's KRAS A59T molecular profile to ask whether codon 59 alterations qualify under the MAPK-pathway eligibility.
46	ST316 (Sapience Therapeutics beta-catenin / BCL9 disruptor)	peptide_antagonist	Clinical trial	Investigational. Phase 1/2 first-in-human.	Email or call 914-418-5100 with the patient's APC E1295 variant call and ask whether MCR-region truncating mutations are accepted into the CRC expansion cohort.
47	JYP0015 (Guangzhou JOYO pan-RAS inhibitor)	small_molecule	Not yet accessible	Investigational. Phase 1/2 in China only.	Do not pursue. Route the pan-RAS question through daraxonrasib (RMC-GI-102) or JAB-23E73.
48	Adagrasib (Krazati) ± cetuximab (KRYSTAL-1, KRAS G12C mCRC)	small_molecule_plus_mab	Unavailable	Adagrasib FDA-approved December 2022 (KRAS G12C NSCLC); accelerated approval June 2024 (KRYSTAL-1) for adagrasib + cetuximab in KRAS G12C mCRC. Allele-specific.	Skip.
49	Adagrasib + pembrolizumab (KRYSTAL-7, KRAS G12C NSCLC)	small_molecule_plus_mab	Unavailable	Investigational combination. Allele-specific G12C target.	Skip.

50	Atezolizumab + cobimetinib (IMblaze370)		Unavailable	Each drug FDA-approved separately. The atezolizumab + cobimetinib mCRC combination was not approved; IMblaze370 failed primary endpoint vs regorafenib in 3L MSS-CRC.	Skip.
51	Atypical KRAS / NRAS variant functional characterization	biology	Unavailable	n/a — biology / characterization row.	Route to the pan-RAS / RAS(ON) drug rows for action.
52	Encorafenib + cetuximab (Braftovi + Erbitux, BEACON)	small_molecule_plus_mab	Unavailable	FDA-approved April 2020 (BEACON CRC) for BRAF V600E-mutated mCRC after one prior line of therapy. FDA-approved December 2024 (BREAKWATER) for 1L BRAF V600E mCRC. Indication is BRAF V600E-specific.	Confirm BRAF V600E status from the comprehensive NGS (target_validation essential row). If V600E is positive AND the team wants to consider this regimen despite the A59T anti-EGFR off-label question, route through the sponsor for guidance; otherwise skip.
53	Eprenetapopt (APR-246, PRIMA-1-MET)	small_molecule	Unavailable	Investigational; development program at Aprea Therapeutics was substantially de-emphasized after the phase 3 MDS trial (NCT03745716) failed in 2020. Aprea pivoted toward ATR / WEE1 inhibitors. No active enrollment for TP53 R273 solid-tumor cohorts.	Treat as a closed door for this patient. The TP53 R273 question moves to synthetic-lethal ATR-class agents (see ATR row).
54	Galunisertib (TGFbR1 inhibitor) + nivolumab	small_molecule_plus_mab	Unavailable	Galunisertib clinical program discontinued by Lilly after the HCC phase 2 readout. No active enrollment in mCRC.	Skip. The TGF-beta-axis question is not actionable today; preserve the rationale for future trial opportunities.

55	Navitoclax (ABT-263)	small_molecule	Unavailable	Investigational; no FDA approval. AbbVie discontinued solid-tumor development after thrombocytopenia limited clinical utility. Myelofibrosis program (navitoclax + ruxolitinib, TRANSFORM-1) failed primary endpoint 2024.	Skip. The BCL-xL-axis question routes through DT2216 or AZD0466.
56	RMC-7977 (Revolution Medicines RAS(ON) tool compound)	small_molecule	Unavailable	Preclinical tool compound. Not in clinical development.	Treat RMC-7977 as preclinical evidence supporting daraxonrasib mechanism; route patient access through the daraxonrasib (RMC-6236) row.
57	Rezatapopt (PC14586, PYNNAACLE)	small_molecule	Unavailable	Investigational. PMV Pharmaceuticals Phase 1/2 PYNNAACLE (NCT04585750) Y220C-restricted; FDA Fast Track designation for TP53 Y220C solid tumors.	Skip. Confirm via target_validation tp53-y220c-ruleout row that there is no co-occurring Y220C variant; if confirmed, this row closes formally.
58	SMAD4 R361H biology — prognostic / no targeted therapy	biomarker	Unavailable	n/a — biomarker / biology row.	Resolve CMS class from the transcriptomic or IHC-surrogate panel (target_validation cms-class-cdx2-mesenchymal row). If CMS4, the TGF-beta-axis story becomes load-bearing for combination ICI trial selection.
59	Sotorasib + ICI combinations (CodeBreaK 101)	small_molecule_plus_mab	Unavailable	Sotorasib + ICI combinations are investigational (CodeBreaK 101 platform). Sotorasib monotherapy is FDA-approved for KRAS G12C NSCLC and CRC; ICI combinations are exploratory. A59T not in scope.	Skip.

60	Sotorasib + panitumumab (CodeBreaK 300, KRAS G12C mCRC)	small_molecule_plus_mab	Unavailable	Sotorasib + panitumumab FDA-approved January 2025 (CodeBreaK 300) for KRAS G12C-mutated mCRC after fluoropyrimidine, oxaliplatin, and irinotecan. Indication is specifically G12C; A59T is not in scope.	Skip. Route the KRAS question through pan-RAS / RAS(ON) inhibitors (daraxonrasib, JAB-23E73) instead.
61	WNT974 (LGK974) porcupine inhibitor	small_molecule	Unavailable	Investigational. Novartis Phase 1 (NCT01351103) completed. The program substantially de-prioritized after dose-limiting bone toxicity; no current active solid-tumor enrollment.	Skip. ST316 is the active Wnt-axis lane.

Standard of care (18)

1. Aflibercept (Zaltrap) + FOLFIRI

Aliases: aflibercept, Zaltrap, ziv-aflibercept, Eylea HD (ophthalmic; not Zaltrap)

Modality: fusion_protein · **Regulatory:** FDA-approved August 2012 (VELOUR) for metastatic CRC progressing after oxaliplatin, in combination with FOLFIRI. EMA-approved with comparable wording. · **Guidelines:** NCCN category 1 second-line option in combination with FOLFIRI after oxaliplatin failure. · **Geographic scope:** Universal availability. · **Verified:** 2026-05-29

Approved, labeled second-line option after FOLFOX/oxaliplatin failure. The patient is oxaliplatin-naïve on FOLFIRI-bev, so aflibercept fits the second-line slot only after a future FOLFOX leg.

Next steps

1. Park aflibercept as a labeled second-line antiangiogenic if the patient eventually switches to FOLFOX and progresses; not in immediate scope while FOLFIRI-bev is active.

Manufacturer / sponsor contact

- **Company:** Regeneron / Sanofi (Sanofi handles oncology marketing as Zaltrap)
- **Med info phone:** 1-800-633-1610
- **Product info:** <https://www.zaltrap.com/>
- **Notes:** Sanofi US medical information 1-800-633-1610 handles Zaltrap. Distinct from Eylea HD which is the

ophthalmic formulation.

Payer / coverage: On-label in second line after oxaliplatin. The patient has not had oxaliplatin yet, so the labeled sequence does not apply unless they advance to triplet-then-aflibercept later. Off-label use as a bevacizumab alternative in 1L is not standard.

Notes: VELOUR is the registrational evidence. Sequence-dependent fit.

2. Bevacizumab (Avastin) and biosimilars (Mvasi, Zirabev, Vegzelma, Alymsys, Abevmy, Aybintio)

Aliases: bevacizumab, Avastin, Mvasi, Zirabev, ABP 215, PF-06439535

Modality: monoclonal_antibody · **Regulatory:** FDA-approved 2004 for first- and second-line mCRC in combination with fluoropyrimidine-based chemo. Multiple biosimilars approved (Mvasi 2017, Zirabev 2019, Vegzelma 2022, Alymsys 2022, Aybintio 2022, Abevmy 2024). EMA-approved with comparable wording. ·

Guidelines: NCCN category 1 component of first- and second-line mCRC regimens. Currently in the patient's regimen. · **Geographic scope:** Universal availability. · **Verified:** 2026-05-29

Active component of the patient's regimen. Hold bevacizumab 4-6 weeks before cytoreductive surgery to limit wound-healing complications; resume 4-6 weeks after surgery once healing is complete.

Next steps

1. Coordinate the bevacizumab pause with the CRS-HIPEC team: last dose at least 4 weeks before surgery, ideally 6, and no restart until the abdominal wound is healed.
2. If the plan switches the patient to a biosimilar mid-line, document the brand for ctDNA-MRD interpretation continuity.

Manufacturer / sponsor contact

- **Company:** Genentech/Roche (Avastin); Amgen (Mvasi); Pfizer (Zirabev); Celltrion (Vegzelma); Amneal (Alymsys); Samsung Bioepis (Aybintio); Biocon Biologics/Viatris (Abevmy)
- **Med info phone:** 1-800-821-8590
- **Med info email:** med.info@gene.com
- **Product info:** <https://www.gene.com/medical-professionals/medicines/avastin>
- **Compassionate / expanded access:**
<https://www.gene.com/patients/access-and-affordability/compassionate-use>
- **Notes:** Genentech routes mCRC inquiries through Avastin Access Solutions: 1-866-422-2377. Biosimilar substitution is plan-driven; clinical equivalence is established.

Payer / coverage: Routine Medicare and commercial coverage as a first-line CRC biologic. Many plans now mandate biosimilar substitution.

Notes: Bevacizumab is the on-label antiangiogenic in the patient's current regimen. Sequencing around HIPEC is the only practical access question.

3. Capecitabine + bevacizumab maintenance (CAIRO3)

Aliases: cape-bev maintenance, CAIRO3 regimen, capecitabine + bevacizumab

Modality: chemotherapy_plus_biologic · **Regulatory:** Component drugs FDA-approved. Maintenance use is published regimen, not a labeled indication; routinely covered. · **Guidelines:** NCCN-listed maintenance option after first-line induction. · **Geographic scope:** Universal availability. · **Verified:** 2026-05-29

Maintenance option after first-line induction. The patient is on FOLFIRI-bev with planned CRS-HIPEC, so the operational decision is whether to introduce a maintenance phase post-HIPEC rather than continue intermittent triplet.

Next steps

1. Decide on a post-HIPEC maintenance plan with the medical-oncology team. If maintenance is chosen, the cape-bev backbone is the operational standard.
2. Confirm DPYD genotype before starting capecitabine (target_validation has DPYD as essential).

Manufacturer / sponsor contact

- **Company:** Genentech (bevacizumab); Genentech/Roche (Xeloda originally; capecitabine generic since 2013)
- **Med info phone:** Genentech 1-800-821-8590
- **Product info:** <https://www.gene.com/medical-professionals/medicines/xeloda>

Payer / coverage: Routine coverage. Capecitabine PO is patient-friendly maintenance; the DPYD status (see target_validation row) gates the dose.

Notes: CAIRO3 is the registrational maintenance trial. Capecitabine became generic in 2013.

4. Cytoreductive surgery + HIPEC (mitomycin C or oxaliplatin)

Aliases: CRS-HIPEC, cytoreductive surgery + heated intraperitoneal chemotherapy, mitomycin HIPEC, oxaliplatin HIPEC

Modality: surgical_procedure · **Regulatory:** Surgical procedure plus off-label intraperitoneal use of approved chemo drugs. Not a labeled drug indication; HIPEC delivery is institution-specific protocol. · **Guidelines:** NCCN now lists CRS + HIPEC as a treatment option for selected mCRC patients with limited peritoneal disease (PCI < 20) at experienced centers (NCCN Colon Cancer v3.2026). ESMO 2025 retains a Category B recommendation for high-volume centers. · **Geographic scope:** United States — multiple high-volume peritoneal surface malignancy centers. · **Verified:** 2026-05-29

Patient already has CRS-HIPEC planned. Access is about choosing a high-volume center and the HIPEC drug. Major US peritoneal surface malignancy centers include MSK, MD Anderson, Mayo Clinic (Rochester, Phoenix, Jacksonville), City of Hope, Wake Forest, UPMC (Pittsburgh's Sugarbaker network), Cleveland Clinic, and Moffitt. Mitomycin C is the post-PRODIGE-7 default; oxaliplatin HIPEC is largely off the table for CRC.

Next steps

1. Confirm the CRS-HIPEC center is high-volume (>20 cases/year) and uses mitomycin C as the default HIPEC

agent; the post-PRODIGE-7 consensus is that oxaliplatin HIPEC adds toxicity without OS benefit.

2. Document peritoneal cancer index (PCI) and CC-0/CC-1 cytoreduction probability at the surgical consult.
3. Hold bevacizumab 4-6 weeks pre-op and resume 4-6 weeks post-op once the wound is healed.
4. Bracket the procedure with ctDNA (Signatera) per the target_validation row; baseline pre-op, day 30-60 post-op, then q8-12 weeks.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04861558		recruiting	unconfirmed	Peter Cashin, MD, PhD; peter.cashin@surgsci.uu.se; +46 18 6174304
NCT04729554		recruiting	likely	Prakash Pandalai, MD; Prakash.Pandalai@uky.edu; 859-323-8920
NCT07291180		recruiting	likely	—

Manufacturer / sponsor contact

- **Company:** Multiple HIPEC pump manufacturers (Belmont Medical, ThermoChem, RanD); chemo drugs are generic
- **Notes:** The procedure is institution-driven, not a single sponsor program. Mitomycin C generics (Mylan, Sandoz) and oxaliplatin generics dominate.

Payer / coverage: Medicare and most commercial payers cover CRS-HIPEC for selected CRC patients at experienced centers; PRODIGE-7 reduced enthusiasm for oxaliplatin HIPEC, and mitomycin C is the default agent in most US centers. Pre-authorization typically requires PCI documentation and a multidisciplinary tumor board record.

Notes: PRODIGE-7 (Quenet et al. Lancet Oncol 2021) showed no OS benefit from oxaliplatin HIPEC. COLOPEC studied adjuvant HIPEC in T4/perforated colon; the EFFIPEC and HIPECT4 trials are the contemporary phase 3 question marks.

5. FOLFIRI (5-FU + leucovorin + irinotecan)

Aliases: FOLFIRI, 5-FU/LV/irinotecan, BICC-C regimen

Modality: chemotherapy · **Regulatory:** Each component FDA-approved decades ago: irinotecan (Camptosar, 1996), 5-FU (1962), leucovorin (1952). All generic; FOLFIRI is a generic combination regimen. · **Guidelines:** NCCN category 1 first-line preferred regimen for metastatic colorectal cancer (NCCN Colon Cancer v3.2026). Patient is on this now. · **Geographic scope:** Universal availability. · **Verified:** 2026-05-29

FOLFIRI is the regimen the patient is currently receiving; access is fully routine. The peri-HIPEC sequencing question matters more than the access question: most centers complete 4-6 cycles, restage, and operate during a 3-6 week washout.

Next steps

1. Confirm UGT1A1 *28 genotype and DPYD status before any dose change (target_validation has these as essential rows).
2. Coordinate the systemic-to-surgical handoff with the HIPEC surgeon so the last irinotecan dose lands at least 3 weeks before CRS to limit perioperative neutropenia.

Manufacturer / sponsor contact

- **Company:** Multiple generic manufacturers (Hospira/Pfizer, Sandoz, Teva, Fresenius Kabi, Sun)
- **Med info phone:** Pfizer 1-800-438-1985 for Camptosar branded inquiries
- **Product info:** <https://www.accessdata.fda.gov/scripts/cder/daf/>
- **Notes:** Irinotecan reference label is Pfizer's Camptosar; 5-FU and leucovorin have no single reference brand. Sourced from any oncology pharmacy.

Payer / coverage: Universal Medicare / commercial coverage as a first-line CRC regimen. No prior auth typically needed beyond standard chemo billing. UGT1A1 28/28 carriers warrant a starting-dose reduction per the irinotecan label.

Notes: BICC-C established FOLFIRI as the irinotecan-backbone reference. Routine first-line option in mCRC.

6. FOLFOX + bevacizumab (mFOLFOX6-bev)

Aliases: FOLFOX-bev, mFOLFOX6-bev, MAVERICC regimen

Modality: chemotherapy_plus_biologic · **Regulatory:** Oxaliplatin FDA-approved 2002; 5-FU, leucovorin, bevacizumab approved; biosimilar bevacizumab options Mvasi / Zirabev approved. · **Guidelines:** NCCN category 1 first-line preferred regimen for mCRC. Equivalent efficacy to FOLFIRI-bev in MAVERICC. · **Geographic scope:** Universal availability. · **Verified:** 2026-05-29

Alternative first-line backbone if FOLFIRI is paused before the HIPEC window or if a different toxicity profile is wanted. Cumulative oxaliplatin neuropathy is the practical limiting factor; the patient has not been exposed to oxaliplatin yet, so this lane is preserved.

Next steps

1. Bank FOLFOX as the post-HIPEC adjuvant option since the patient is oxaliplatin-naïve — saves the cleaner second-line drug for the moment it is most needed.

Manufacturer / sponsor contact

- **Company:** Sanofi (Eloxatin reference) + generic oxaliplatin makers; Genentech/Roche bevacizumab
- **Med info phone:** Sanofi US medical information 1-800-633-1610; Genentech 1-800-821-8590
- **Med info email:** med.info@gene.com
- **Product info:** https://www.accessdata.fda.gov/drugsatfda_docs/label/2015/021759s016lbl.pdf

Payer / coverage: Universal first-line coverage. No prior-auth obstacles.

Notes: MAVERICC and Stein et al. established equivalence between FOLFOX-bev and FOLFIRI-bev in 1L mCRC.

7. FOLFOXIRI + bevacizumab (TRIBE regimen)

Aliases: FOLFOXIRI-bev, TRIBE regimen, 5-FU/LV/oxaliplatin/irinotecan/bevacizumab

Modality: chemotherapy_plus_biologic · **Regulatory:** Component drugs all FDA-approved. The FOLFOXIRI combination is a published regimen rather than a labeled product; bevacizumab is on-label as a first-line mCRC combination partner (Avastin label). · **Guidelines:** NCCN category 1 first-line option for patients with good performance status, especially with high tumor burden, peritoneal disease, or interest in cytoreduction. · **Geographic scope:** Universal availability. · **Verified:** 2026-05-29

An intensification option if the team wants to expand the regimen from FOLFIRI + bev to triplet + bev before CRS-HIPEC. Coverage is routine; the operational decision is tolerability and the peri-operative window, not access.

Next steps

1. Reassess whether triplet intensification is wanted given the patient is already tolerating FOLFIRI-bev. CAIRO6 is the protocol-shaped template even though it is closed to new enrollment.
2. If switching, confirm oxaliplatin-naïve status (the prior-therapy list shows FOLFIRI only) and lay in a neuropathy plan.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02722351		active not recruiting	likely	—

Manufacturer / sponsor contact

- **Company:** Component generics + Genentech/Roche (bevacizumab)
- **Med info phone:** Genentech medical information 1-800-821-8590
- **Med info email:** med.info@gene.com
- **Product info:** <https://www.gene.com/medical-professionals/medicines/avastin>
- **Notes:** FOLFOXIRI components all generic. Bevacizumab is the on-label biologic in the regimen; Avastin / biosimilars Mvasi (Amgen) and Zirabev (Pfizer) are routinely substituted.

Payer / coverage: Standard coverage. TRIBE / TRIBE2 are the registrational evidence for adding oxaliplatin and irinotecan together in 1L; routinely approved for younger fit patients with peritoneal-dominant or high-tumor-burden disease.

Notes: TRIBE / TRIBE2 anchor the evidence base. CAIRO6 is the cleanest peritoneal-spread protocol template even though new enrollment is closed.

8. FoundationOne Tracker (Foundation Medicine tumor-informed ctDNA MRD)

Aliases: FoundationOne Tracker

Modality: diagnostic_assay · **Regulatory:** LDT; Medicare-covered under MoDx LCD. CLIA-certified. · **Guidelines:** NCCN-acknowledged ctDNA tool. · **Geographic scope:** United States. · **Verified:** 2026-05-29

Tumor-informed assay; useful when FoundationOne CDx is the master NGS panel.

Next steps

- 1. Use only if FoundationOne CDx is the master NGS panel; otherwise default to Signatera.

Manufacturer / sponsor contact

- **Company:** Foundation Medicine (Roche)
- **Med info phone:**888-988-3639
- **Med info email:**client.services@foundationmedicine.com
- **Product info:**<https://www.foundationmedicine.com/test/foundationone-tracker>

Payer / coverage: Medicare-covered.

Notes: Pairs with FoundationOne CDx tissue NGS.

9. Fruquintinib (Fruzaqla)

Aliases: fruquintinib, Fruzaqla, HMPL-013

Modality: small_molecule · **Regulatory:** FDA-approved November 8, 2023 (FRESCO-2) for previously treated metastatic CRC after fluoropyrimidine, oxaliplatin, irinotecan, anti-VEGF, and (if RAS-WT) anti-EGFR therapy. EMA-approved June 2024. · **Guidelines:** NCCN category 1 third-line+ option for mCRC after standard chemo and biologic exposure. · **Geographic scope:** US, EU, UK, AU available; China-specific trials are domestic. · **Verified:** 2026-05-29

Approved 3L+ option. The patient is still 1L on FOLFIRI-bev so fruquintinib is not yet in scope, but bank it as a labeled third-line lane.

Next steps

- 1. Reserve fruquintinib as the post-FOLFOX, post-bevacizumab labeled antiangiogenic for 3L use.
- 2. If the patient eventually reaches 3L, Takeda's Here2Assist (1-844-817-6468) handles benefits investigations and copay support.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06980232	2B/32	recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Takeda (US and ex-China commercial rights); HUTCHMED (China)
- **Med info phone:**1-844-662-8532
- **Product info:**<https://www.fruzaqlahcp.com/>
- **Compassionate / expanded access:**
<https://www.takeda.com/what-we-do/our-clinical-trials/expanded-access-program/>

- **Notes:** Takeda Oncology Medical Information operates 1-844-662-8532. Patient support: Takeda Oncology Here2Assist at 1-844-817-6468.

Payer / coverage: Routine third-line coverage. Will become relevant for this patient only after at least 2 prior lines (FOLFIRI-bev, then FOLFOX or another biologic). Manageable hypertension is the main toxicity gating; bevacizumab co-administration is not standard.

Notes: FRESCO-2 was the registrational global trial; FRESCO was the China-only precursor.

10. Guardant Reveal (Guardant Health tumor-naive ctDNA MRD)

Aliases: Guardant Reveal, Reveal

Modality: diagnostic_assay · **Regulatory:** Laboratory-developed test (LDT). Medicare-covered under MoDx for CRC MRD surveillance. CLIA-certified. · **Guidelines:** NCCN-acknowledged ctDNA tool alongside Signatera. · **Geographic scope:** United States. · **Verified:** 2026-05-29

Tumor-naive ctDNA MRD alternative to Signatera. No bespoke build needed (faster to first result) but lower sensitivity at low tumor fraction. Reasonable choice if the FFPE block is unavailable for Signatera build.

Next steps

1. Default to Signatera for the peri-HIPEC plan (target_validation prefers Signatera). Reserve Guardant Reveal as the fallback if no tissue is available.

Manufacturer / sponsor contact

- **Company:** Guardant Health
- **Med info phone:** 855-698-8887
- **Med info email:** info@guardanthealth.com
- **Product info:** <https://guardanthealth.com/products/guardant-reveal/>
- **Notes:** Guardant order portal handles physician requisitions.

Payer / coverage: Medicare-covered; commercial coverage follows Medicare in most states.

Notes: Tumor-naive assay; faster turnaround.

11. Low-dose aspirin (PIK3CA-mutant secondary-prevention rationale, ALASCCA)

Aliases: aspirin, ASA, acetylsalicylic acid

Modality: small_molecule · **Regulatory:** Over-the-counter; FDA-approved for cardiovascular prevention. Anti-cancer use is off-label. · **Guidelines:** Not yet in NCCN as adjuvant CRC therapy. ALASCCA (NEJM 2025) is the registrational evidence for PIK3CA-mutant CRC adjuvant aspirin. ASCO has not endorsed routine adjuvant aspirin yet pending follow-up trials. · **Geographic scope:** Universal availability. · **Verified:** 2026-05-29

OTC product. The PIK3CA-mutant adjuvant-aspirin rationale comes from ALASCCA (Martinez Bedoya et al. NEJM

2025), which showed a recurrence reduction in PIK3CA-altered resected CRC on low-dose aspirin. Off-label as cancer therapy but trivially accessible. The decision is whether to add it; bleeding risk in the peri-HIPEC window is the main concern.

Next steps

- 1. Discuss low-dose (81 mg) aspirin with the medical oncology team based on the ALASCCA PIK3CA-mutant signal.
- 2. Coordinate aspirin start / hold with the CRS-HIPEC surgeon — most teams stop aspirin 5-7 days pre-op and resume on POD 1-3.
- 3. Document baseline GI bleeding risk and any anticoagulation overlap.

Manufacturer / sponsor contact

- **Company:** Bayer (Bayer Aspirin); multiple generic manufacturers
- **Med info phone:** 1-800-331-4536
- **Product info:** <https://www.bayer.com/>
- **Notes:** OTC product; pharmacy access.

Payer / coverage: OTC; no payer involvement. Cost is negligible (~\$0.05/pill at any pharmacy).

Notes: ALASCCA randomized resected stage I-III PIK3CA-altered CRC patients to aspirin vs placebo; recurrence reduction was meaningful. Not yet guideline-endorsed for metastatic disease.

12. Perioperative systemic chemo + CRS-HIPEC (CAIRO6 schema)

Aliases: perioperative systemic + CRS-HIPEC, CAIRO6 schema

Modality: treatment_strategy · **Regulatory:** Strategy combining approved drugs and standard surgical procedure. CAIRO6 is the registrational trial. · **Guidelines:** Multidisciplinary practice in high-volume centers; not a single-drug labeled indication. · **Geographic scope:** Universal availability at high-volume centers. · **Verified:** 2026-05-29

This is essentially the patient's pathway: induction FOLFIRI-bev, restaging, CRS-HIPEC, post-op systemic therapy. CAIRO6 is the closest published protocol template; new enrollment closed but the schema is operational at every high-volume US peritoneal-malignancy center.

Next steps

- 1. Coordinate the systemic-to-surgical-to-systemic transition with the medical oncology, surgical oncology, and HIPEC teams at the chosen center.
- 2. Use CAIRO6 as the published protocol template even though enrollment is closed.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02728351	2/3	active not recruiting	no	Koen Rovers / Ignace de Hingh

NCT06733491	not yet recruiting	unconfirmed	—
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Manufacturer / sponsor contact

- **Company:** Multi-sponsor; surgical pathway
- **Notes:** Strategy uses generic / branded chemo drugs already in scope (FOLFOX, FOLFOXIRI, bevacizumab).

Payer / coverage: Standard coverage for the constituent drugs and surgery. The strategy is what the patient is already on.

Notes: CAIRO6 design (FOLFOX or FOLFOXIRI ± bev before CRS-HIPEC) is the contemporary protocol reference.

13. Ramucirumab (Cyramza) + FOLFIRI

Aliases: ramucirumab, Cyramza, IMC-1121B, LY3009806

Modality: monoclonal_antibody · **Regulatory:** FDA-approved April 2015 (RAISE) for second-line mCRC in combination with FOLFIRI after bevacizumab + oxaliplatin + fluoropyrimidine. EMA-approved with similar wording.
 · **Guidelines:** NCCN category 1 second-line option (FOLFIRI + ramucirumab) after first-line bev-containing oxaliplatin regimens. · **Geographic scope:** Universal availability. · **Verified:** 2026-05-29

Approved second-line antiangiogenic. RAISE specifically tested it after FOLFOX-bev, so its label is sequence-dependent. The patient is on FOLFIRI-bev in 1L, so ramucirumab fits the second-line slot if they advance through oxaliplatin first.

Next steps

1. Hold as a sequence option for 2L+ after oxaliplatin exposure. No immediate action.

Manufacturer / sponsor contact

- **Company:** Eli Lilly
- **Med info phone:** 1-800-545-5979
- **Product info:** <https://www.cyramza.com/hcp>
- **Compassionate / expanded access:** <https://www.lilly.com/policies-positions/expanded-access>
- **Notes:** Lilly Medical Information operates 1-800-LillyRx for HCP queries. Expanded-access is case-by-case.

Payer / coverage: Labeled second-line use; routine Medicare coverage. Same patient-specific caveat as aflibercept: the FOLFIRI partner is on-label but RAISE enrolled patients post-FOLFOX-bev.

Notes: RAISE registrational. Comparable to aflibercept on hazard ratio.

14. Regorafenib (Stivarga)

Aliases: regorafenib, Stivarga, BAY 73-4506

Modality: small_molecule · **Regulatory:** FDA-approved 2012 for refractory mCRC. ReDOS dose-escalation (80→120→160 mg) is the operational standard. · **Guidelines:** NCCN category 1 third-line+ option, with ReDOS escalation noted as a tolerability framework. · **Geographic scope:** Universal availability. · **Verified:** 2026-05-29

Approved 3L+ option, will become operationally relevant later in the disease course. Same as fruquintinib: not in scope until the patient progresses past 2L.

Next steps

1. Bank as a 3L+ antiangiogenic. REGONIVO (regorafenib + nivolumab) is the MSS-CRC immune-combination precedent but did not move the needle in confirmatory trials.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05672216		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Bayer
- **Med info phone:** 1-888-842-2937
- **Med info email:** medinfo@bayer.com
- **Product info:** <https://www.stivargahcp.com/>
- **Compassionate / expanded access:** <https://pharma.bayer.com/expanded-access-and-compassionate-use>
- **Notes:** Bayer Medical Information (1-888-84-BAYER) handles Stivarga inquiries. REACH Patient Assistance program for uninsured patients.

Payer / coverage: Routine 3L+ coverage. ReDOS dosing is increasingly the default to manage hand-foot-skin reaction.

Notes: CORRECT and CONCUR were the registrational trials. ReDOS established the dose-escalation framework.

15. Signatera (Natera tumor-informed ctDNA MRD)

Aliases: Signatera, Natera ctDNA MRD

Modality: diagnostic_assay · **Regulatory:** Laboratory-developed test (LDT). Medicare-covered for stage II-IV CRC MRD surveillance under MoDx LCD. CLIA-certified at Natera. · **Guidelines:** NCCN colon v3.2026 acknowledges ctDNA as a developing prognostic tool. Signatera is the most-cited tumor-informed assay. · **Geographic scope:** United States. · **Verified:** 2026-05-29

Routinely accessible. Standard ctDNA MRD assay for the peri-HIPEC surveillance plan described in target_validation. Order baseline pre-CRS, post-op day 30-60, then every 8-12 weeks during surveillance.

Next steps

1. Order Signatera baseline pre-CRS-HIPEC; Natera will design the bespoke 16-variant panel from the FFPE

block.

2. Bracket the procedure: baseline, day 30-60 post-op, then every 8-12 weeks.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05124369	2/3	recruiting	no	Judy Langer; langerj@nrgoncology.org; 412-339-5300

Manufacturer / sponsor contact

- **Company:** Natera
- **Med info phone:** 650-249-9090
- **Med info email:** support@natera.com
- **Product info:** <https://www.natera.com/oncology/signatera-advanced-cancer-detection/>
- **Notes:** Natera oncology support: 650-249-9090. Order Signatera through the standard NaceGo / EMR ordering portal.

Payer / coverage: Medicare-covered for stage II-IV CRC MRD surveillance (MoIDx LCD L38779). Most commercial payers follow Medicare; Natera's billing team handles patient-specific authorization at no balance-billing for Medicare patients.

Notes: Tumor-informed bespoke panel from WES. The clinical default for CRC MRD.

16. Tempus xM (Tempus tumor-informed ctDNA MRD)

Aliases: Tempus xM

Modality: diagnostic_assay · **Regulatory:** LDT; Medicare-covered for select CRC MRD indications. ·

Guidelines: NCCN-acknowledged ctDNA tool. · **Geographic scope:** United States. · **Verified:** 2026-05-29

Tumor-informed alternative to Signatera, useful if the patient's NGS is already being routed through Tempus xT.

Next steps

1. Use only if Tempus xT is the master NGS panel; otherwise default to Signatera.

Manufacturer / sponsor contact

- **Company:** Tempus Labs
- **Med info phone:** 800-739-4137
- **Med info email:** support@tempus.com
- **Product info:** <https://www.tempus.com/oncology/genomic-profiling/xm/>

Payer / coverage: Medicare coverage variable by region; commercial coverage growing.

Notes: Operationally cleanest when the master NGS is on the Tempus platform.

17. Trifluridine/tipiracil (Lonsurf, TAS-102)

Aliases: trifluridine/tipiracil, TAS-102, Lonsurf, FTD/TPI

Modality: small_molecule · **Regulatory:** FDA-approved September 2015 (RECOURSE) for refractory mCRC. Label updated 2023 (SUNLIGHT) to include the trifluridine/tipiracil + bevacizumab combination. · **Guidelines:** NCCN category 1 third-line+ option, both as monotherapy and in combination with bevacizumab (SUNLIGHT regimen). · **Geographic scope:** Universal availability. · **Verified:** 2026-05-29

Approved 3L+ option. SUNLIGHT made the bevacizumab-combination an on-label addition. Reserved for the post-FOLFIRI / post-FOLFOX setting; not in scope while the patient is 1L.

Next steps

1. Bank as a 3L+ option, paired with bev under the SUNLIGHT regimen.
2. If the patient continues bevacizumab through multiple lines (no Avastin holiday between regimens), confirm cumulative dose tolerability with the prescribing physician.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06522919		recruiting	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** Taiho Oncology
- **Med info phone:** 1-844-878-2446
- **Product info:** <https://www.lonsurfhcp.com/>
- **Compassionate / expanded access:** <https://www.taihooncology.com/us/contact/>
- **Notes:** Taiho Oncology Medical Information at 1-844-US-TAIHO (1-844-878-2446). Patient support: Taiho Oncology Patient Support at 1-844-824-4648.

Payer / coverage: Routine 3L+ coverage as monotherapy or in combination with bevacizumab.

Notes: RECOURSE established 3L monotherapy; SUNLIGHT extended the label to the bev-combination.

18. Trifluridine/tipiracil + bevacizumab (SUNLIGHT)

Aliases: TAS-102 + bev, SUNLIGHT regimen, FTD/TPI + bevacizumab

Modality: chemotherapy_plus_biologic · **Regulatory:** FDA label updated August 2023 to include the trifluridine/tipiracil + bevacizumab combination (SUNLIGHT). Both components individually approved. · **Guidelines:** NCCN category 1 third-line option after fluoropyrimidine, oxaliplatin, irinotecan, and prior bev exposure. · **Geographic scope:** Universal availability. · **Verified:** 2026-05-29

Labeled 3L+ option that meaningfully extends OS over TAS-102 monotherapy (SUNLIGHT). Operational fit is 3L+, not now.

Next steps

1. Bank for the 3L+ sequence after FOLFIRI-bev and FOLFOX exposure.

Manufacturer / sponsor contact

- **Company:** Taiho Oncology (TAS-102) + Genentech/Roche (bevacizumab)
- **Med info phone:** Taiho 1-844-878-2446; Genentech 1-800-821-8590
- **Product info:** <https://www.lonsurfhcp.com/>
- **Notes:** Same contacts as the monotherapy and bevacizumab rows.

Payer / coverage: Labeled combination; routinely covered. Cumulative bev exposure is the practical concern across multiple lines.

Notes: SUNLIGHT N Engl J Med 2023 anchors the combination evidence.

Off-label use (13)

19. Alpelisib (Vijoice / Piqray, BYL719)

Aliases: alpelisib, Piqray, Vijoice, BYL719

Modality: small_molecule · **Regulatory:** FDA-approved May 2019 (Piqray, SOLAR-1) for PIK3CA-mutant HR+/HER2- advanced breast cancer in combination with fulvestrant. FDA-approved April 2022 (Vijoice) for PIK3CA-related overgrowth spectrum (PROS) in pediatric and adult patients. EMA-approved. · **Guidelines:** On-label only in breast and PROS. Not in NCCN Colon v3.2026. Off-label use in PIK3CA-mutant mCRC has no compendium support. · **Geographic scope:** US, EU on-label in breast; off-label everywhere else. · **Verified:** 2026-05-29

Approved in breast and PROS; off-label in mCRC with no compendium support. The only mCRC trial (ALCAP) is closed in Korea, so there is no clean US trial path. Route the PI3K-alpha question toward inavolisib via INTRINSIC instead of pursuing alpelisib off-label.

Next steps

1. Default toward inavolisib (INTRINSIC, NCT04929223) for the PI3K-alpha axis; alpelisib does not have a US trial slot for mCRC.
2. If alpelisib is specifically wanted (e.g., as the more-studied PI3K-alpha agent), Novartis MAP (compassionate access) can be approached at compassionate.use@novartis.com but expect declines for mCRC requests.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04751203		unknown	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** Novartis (Piqray and Vioice rights to Novartis; Vituvia is the Vioice tradename in some markets)
- **Med info phone:** 1-888-669-6682
- **Med info email:** medinfo.us@novartis.com
- **Product info:** <https://www.hcp.novartis.com/products/piqray/>
- **Compassionate / expanded access:**
<https://www.novartis.com/research-development/clinical-trials/managed-access-programs-faqs>
- **Notes:** Novartis Medical Information operates 1-888-NOW-NOVA. Patient Assistance Foundation handles uninsured; pre-approval Managed Access Program (MAP) handles compassionate / expanded access.

Payer / coverage: Off-label in mCRC. Hyperglycemia management is the practical limiter; the drug is not popular outside breast. Medical-exception requests citing the PIK3CA M1043I activating call typically fail without trial coverage.

Notes: SOLAR-1 anchored the breast approval. Long-term hyperglycemia and rash limit the practical mCRC use case.

20. Anti-EGFR mAbs (cetuximab / panitumumab) in extended-RAS-mutant mCRC — evidence-summary entry

Aliases: anti-EGFR atypical RAS, cetuximab atypical KRAS, panitumumab atypical KRAS

Modality: monoclonal_antibody · **Regulatory:** Composite category; see cetuximab and panitumumab rows. · **Guidelines:** Atypical RAS alleles (including codon 59) are treated as anti-EGFR resistance variants in NCCN v3.2026 and ESMO 2025. Single case reports are not guideline-supported. · **Geographic scope:** Off-label everywhere. · **Verified:** 2026-05-29

Aggregated row for the cetuximab/panitumumab + atypical-KRAS evidence base. Bottom line: treat anti-EGFR as off the table for KRAS A59T outside a sponsor-coverage trial (RMC-GI-102) or a documented medical-exception with a low-VAf rationale.

Next steps

1. Use the cetuximab and panitumumab rows for operational decisions; this row captures the meta-evidence summary.

Manufacturer / sponsor contact

- **Company:** Lilly (cetuximab); Amgen (panitumumab)
- **Notes:** See cetuximab and panitumumab rows for direct contact information.

Payer / coverage: Off-label in atypical KRAS alleles. The collected evidence (meta-analysis Sorich et al., De Roock et al., case reports) supports treating extended-RAS-mutant tumors as anti-EGFR resistant rather than sensitive.

Notes: Meta-analysis and retrospective cohorts (Sorich, De Roock, Bando, Cremolini) anchor the resistance call. The single 2018 case report is the dissenting datum.

21. Capivasertib (Truqap, AZD5363)

Aliases: capivasertib, Truqap, AZD5363

Modality: small_molecule · **Regulatory:** FDA-approved November 2023 (CAPItello-291) for PIK3CA / AKT1 / PTEN-altered HR+/HER2- advanced breast cancer in combination with fulvestrant. EMA-approved 2024. ·

Guidelines: On-label only in breast. Not in NCCN Colon v3.2026. · **Geographic scope:** US, EU on-label in breast; off-label everywhere else. · **Verified:** 2026-05-29

Approved in breast; off-label in mCRC with no active US trial slot for colorectal disease. The PI3K-axis question for this patient routes more cleanly through inavolisib (INTRINSIC) than through capivasertib.

Next steps

1. Do not pursue off-label outside a trial. AKT-inhibitor activity in PIK3CA-mut mCRC is mechanistically rational but clinically thin.

Manufacturer / sponsor contact

- **Company:** AstraZeneca
- **Med info phone:** 1-800-236-9933
- **Product info:** <https://medicalinformation.astrazeneca-us.com/home/prescribing-information/truqap-tablets.html>
- **Compassionate / expanded access:** <https://www.azandmeapp.com/>
- **Notes:** AstraZeneca Medical Information operates 1-800-236-9933. AZ Access 360 patient support: 1-844-275-2360 (1-844-ASK-A360).

Payer / coverage: Off-label in mCRC. AKT1 E17K mutations were the historical biomarker for capivasertib activity in earlier phase 2 work; without an active mCRC trial slot, payer pushback for off-label use is universal.

Notes: CAPItello-291 is the registrational breast trial.

22. Celecoxib (Celebrex) — PIK3CA-mutant adjuvant rationale (CALGB/SWOG 80702)

Aliases: celecoxib, Celebrex, COX-2 inhibitor

Modality: small_molecule · **Regulatory:** FDA-approved 1998 for arthritis pain. Off-label as CRC adjuvant. ·

Guidelines: Not endorsed in NCCN. CALGB/SWOG 80702 did not meet the primary endpoint overall; the PIK3CA-mutant subgroup signal is hypothesis-generating. · **Geographic scope:** Universal availability. ·

Verified: 2026-05-29

Off-label CRC adjuvant rationale that did not survive the CALGB/SWOG 80702 trial overall. The PIK3CA-mutant subgroup signal is interesting but underpowered; aspirin (ALASCCA) is the cleaner adjuvant story for this allele.

Next steps

1. Default to aspirin (ALASCCA) rather than celecoxib for the PIK3CA-axis adjuvant signal. Celecoxib does not add over aspirin in this case.

Manufacturer / sponsor contact

- **Company:** Pfizer (Celebrex; generic since 2014)
- **Med info phone:** 1-800-438-1985
- **Product info:** <https://www.pfizermedical.com/celebrex>

Payer / coverage: Off-label as adjuvant; routine coverage for pain indication. Cardiovascular risk is the practical caveat in younger patients.

Notes: CALGB/SWOG 80702 (Meyerhardt et al.) tested adjuvant celecoxib in resected stage III colon cancer; primary endpoint not met.

23. Cetuximab (Erbix)

Aliases: cetuximab, Erbitux, C225, IMC-C225

Modality: monoclonal_antibody · **Regulatory:** FDA-approved 2004 for KRAS wild-type, EGFR-expressing mCRC (left-sided preferred). The FDA label explicitly excludes KRAS-mutant tumors based on extended-RAS analyses (CRYSTAL, FIRE-3, CALGB/SWOG 80405). EMA-approved with similar exclusion. · **Guidelines:** NCCN category 1 for RAS wild-type, BRAF wild-type, left-sided mCRC in first and later lines. NCCN treats codon 59 mutations as resistance variants alongside the canonical exon 2/3/4 KRAS hotspots. · **Geographic scope:** Universal availability; off-label in this allele. · **Verified:** 2026-05-29

Off-label for this patient because A59T is treated as a RAS-pathway-activating mutation by current guideline consensus. A single 2018 case report (Mavroudis et al., panitumumab in KRAS A59T mCRC) is the entire body of clinical-response data for atypical KRAS responses to anti-EGFR, and the consensus-fit answer is to not use cetuximab off-label in this allele. The clean route into cetuximab is the RMC-GI-102 (NCT06445062) daraxonrasib + cetuximab combination arm where the sponsor pays.

Next steps

1. Treat single-agent or chemo-combination cetuximab as off the table outside a trial; do not bet payer effort on the panitumumab case-report precedent.
2. If the team wants cetuximab in the regimen, route the patient toward RMC-GI-102 (NCT06445062) and ask Revolution Medicines whether A59T is accepted in the daraxonrasib + cetuximab arm.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06445062	1	recruiting	unconfirmed	Revolution Medicines; medinfo@revmed.com ; 1-844-273-8633

Manufacturer / sponsor contact

- **Company:** Eli Lilly (ImClone Systems / Bristol-Myers Squibb originally)
- **Med info phone:** 1-800-545-5979
- **Product info:** <https://www.erbitux.com/hcp>
- **Compassionate / expanded access:** <https://www.lilly.com/policies-positions/expanded-access>

Payer / coverage: Off-label in any RAS-mutant tumor including atypical alleles. Payer denial is essentially universal unless a panitumumab-A59T case-report precedent (pmid:30538852) is cited and the team obtains a medical-exception with sponsor or trial coverage. The cleanest payer-supported access is via the daraxonrasib + cetuximab arm of RMC-GI-102, where the sponsor covers cetuximab.

Notes: FIRE-3 and CALGB/SWOG 80405 anchor the RAS-wild-type cetuximab evidence. A59T is treated as a resistance variant by NCCN.

24. Inavolisib (Itovebi, GDC-0077)

Aliases: inavolisib, Itovebi, GDC-0077, RG6114

Modality: small_molecule · **Regulatory:** FDA-approved October 2024 (INAVO120) for PIK3CA-mutant, HR+, HER2-negative, endocrine-resistant advanced breast cancer in combination with palbociclib and fulvestrant. EMA approval pending. No mCRC approval. · **Guidelines:** On-label only in breast. Not in NCCN Colon v3.2026. Off-label use in PIK3CA-mutant mCRC has no compendium support. · **Geographic scope:** US, EU sites — Genentech platform. · **Verified:** 2026-05-29

Approved in PIK3CA-mutant breast cancer; off-label and not compendium-supported in mCRC. The clean access path for this patient is the Genentech INTRINSIC mCRC platform (NCT04929223), where PIK3CA M1043I would slot into the PI3K-alpha activating-mutation cohort and the sponsor covers the drug. Sponsor confirmation on A59T compatibility is the load-bearing gate.

Next steps

1. Email global-roche-genentech-trials@gene.com or call 888-662-6728 with the PIK3CA M1043I + KRAS A59T molecular profile to ask which INTRINSIC arm fits.
2. Confirm M1043I is treated as kinase-domain-activating by the sponsor's eligibility criteria (the canonical hotspots H1047R/E545K/E542K dominate the registry; M1043I is rarer and may require a sponsor exception).
3. If INTRINSIC is not accessible, do not pursue inavolisib off-label outside a trial — alpelisib has more mCRC-relevant trial signal.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04929223		recruiting	likely	Roche-Genentech Trials Information Hotline; 888-662-6728
NCT04121399		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Genentech / Roche
- **Med info phone:** 1-800-821-8590
- **Med info email:** med.info@gene.com
- **Product info:** <https://www.gene.com/medical-professionals/medicines/itovebi>

- **Compassionate / expanded access:**

<https://www.gene.com/patients/access-and-affordability/compassionate-use>

- **Notes:** Genentech Access Solutions: 1-866-422-2377. Genentech Patient Foundation handles uninsured access; expanded-access program (EAP) is the formal pre-approval channel for off-label requests.

Payer / coverage: Off-label in mCRC; payer support is unlikely without trial coverage. INTRINSIC (NCT04929223) is the cleanest path because Genentech covers the drug under the protocol.

Notes: Selective inhibitor + degrader of mutant p110-alpha. INAVO120 anchored the breast approval. INTRINSIC is the umbrella mCRC platform.

25. Nivolumab + ipilimumab (CheckMate 142, MSI-H mCRC)

Aliases: nivolumab, Opdivo, ipilimumab, Yervoy, CheckMate 142

Modality: monoclonal_antibody_combination · **Regulatory:** FDA-approved July 2018 (CheckMate 142) for MSI-H / dMMR mCRC. CheckMate 8HW expanded the 1L MSI-H combination indication. · **Guidelines:** NCCN category 1 for MSI-H / dMMR mCRC. Not applicable to MSS. · **Geographic scope:** n/a · **Verified:** 2026-05-29

MSI-H / dMMR only. Patient is MSS — not applicable. Listed for explicit rule-out.

Next steps

1. Skip.

Manufacturer / sponsor contact

- **Company:** Bristol Myers Squibb
- **Med info phone:** 1-800-721-5072
- **Product info:** <https://www.opdivohcp.com/>
- **Notes:** BMS Medical Information: 1-800-721-5072.

Payer / coverage: Off-label in MSS. Not covered.

Notes: CheckMate 142 / 8HW.

26. Panitumumab (Vectibix)

Aliases: panitumumab, Vectibix, ABX-EGF

Modality: monoclonal_antibody · **Regulatory:** FDA-approved 2006 for KRAS wild-type mCRC. Label updated 2014 to include extended-RAS wild-type. EMA-approved with similar wording. · **Guidelines:** NCCN category 1 for RAS wild-type, BRAF wild-type, left-sided mCRC. KRAS A59T treated as a resistance variant alongside the canonical hotspots; not a labeled-eligible allele. · **Geographic scope:** Universal availability; off-label in this allele. · **Verified:** 2026-05-29

Off-label for A59T-mutant disease. One published case report (Mavroudis et al. 2018) is the only direct evidence

that an A59T tumor can respond to anti-EGFR. The clean defensive call is to treat anti-EGFR as off the table for this allele; the team can revisit if the A59T VAF is low and a clonal sub-population looks RAS-wild-type on plasma.

Next steps

1. Confirm the KRAS A59T VAF and that the rest of the extended RAS panel (KRAS exon 2/3/4, NRAS, HRAS) and BRAF V600 are wild-type before any anti-EGFR conversation.
2. If the VAF is low and the rest of the panel is clean, run the Mavroudis case report past the patient's payer for a medical-exception request, with the understanding that approval is unlikely.
3. If pursued, Amgen Medical Information (1-800-77-AMGEN) can supply the labeled prescribing information packet but does not advocate for off-label use.

Manufacturer / sponsor contact

- **Company:** Amgen
- **Med info phone:** 1-800-772-6436
- **Med info email:** medinfo@amgen.com
- **Product info:** <https://www.amgenoncology.com/products/vectibix/>
- **Compassionate / expanded access:** <https://www.amgen.com/science/clinical-trials/expanded-access>
- **Notes:** Amgen Medical Information operates 1-800-77-AMGEN for HCP queries. Amgen Assist 360 handles patient access at 1-888-427-7478.

Payer / coverage: Off-label in atypical-RAS tumors. The single supportive case is Mavroudis et al. 2018 (pmid:30538852), which reported a panitumumab-FOLFIRI response in a KRAS A59T-mutant patient. Off-label exception requests can cite this single report but most payers decline. Amgen Assist 360 cannot help unless the patient is on-label.

Notes: The Mavroudis et al. 2018 case report is the only published clinical-response data point in KRAS A59T. NCCN treats codon 59 mutations as anti-EGFR resistance variants.

27. Pembrolizumab (Keytruda) — MMR-deficient mCRC indication

Aliases: pembrolizumab, Keytruda, MK-3475

Modality: monoclonal_antibody · **Regulatory:** FDA-approved May 2017 (tumor-agnostic MSI-H/dMMR); approved 2020 (KEYNOTE-177) for first-line MSI-H/dMMR mCRC. Tumor-agnostic TMB-H (≥ 10 mut/Mb) approval June 2020. · **Guidelines:** NCCN category 1 for MSI-H / dMMR mCRC. Not applicable to MSS / TMB-low (this patient). · **Geographic scope:** n/a for this patient. · **Verified:** 2026-05-29

Patient is MSS / TMB-low / PD-L1-neg per profile.json. Single-agent pembrolizumab is off the table. Listed so the rule-out is documented.

Next steps

1. Skip as monotherapy. The ICI question routes through BOT/BAL combinations (see botensilimab-balstilimab row).

Manufacturer / sponsor contact

- **Company:** Merck
- **Med info phone:** 1-800-672-6372
- **Product info:** <https://www.merckmedicus.com/>
- **Compassionate / expanded access:** <https://www.merckhelps.com/>
- **Notes:** Merck Medical Information operates 1-800-MERCK-RX. Merck Access Program for uninsured patients: 1-866-363-6379.

Payer / coverage: On-label only with MSI-H / dMMR or TMB-H ≥ 10 mut/Mb. The patient is MSS, TMB-low, PD-L1-neg — none of the labeled indications apply. Off-label use without one of these biomarkers is not covered.

Notes: MSS / TMB-low explicitly excludes the labeled indications.

28. Regorafenib + nivolumab (REGONIVO)

Aliases: regorafenib + nivolumab, REGONIVO, EPOC1603

Modality: small_molecule_plus_mab · **Regulatory:** Each drug FDA-approved separately. The combination was not approved for mCRC; the confirmatory trial (NCT04126733) did not move the needle on the Japan-cohort signal. · **Guidelines:** Not in NCCN. · **Geographic scope:** Off-label everywhere. · **Verified:** 2026-05-29

Off-label combination with mixed phase 2 / failed phase 3 confirmation in Western populations. Not a productive lane for this patient — route ICI to BOT/BAL instead.

Next steps

1. Skip. Route ICI to BOT/BAL combinations.

Manufacturer / sponsor contact

- **Company:** Bayer (regorafenib) + BMS (nivolumab)
- **Med info phone:** Bayer 1-888-842-2937; BMS 1-800-721-5072

Payer / coverage: Off-label combination without compendium support. Most payers decline; the Japan-cohort signal did not survive Western confirmation.

Notes: EPOC1603 Japan signal not confirmed in Western trials.

29. Trastuzumab + lapatinib (HERACLES regimen)

Aliases: trastuzumab + lapatinib, Tykerb + Herceptin, HERACLES

Modality: small_molecule_plus_mab · **Regulatory:** Each component FDA-approved (trastuzumab 1998; lapatinib 2007 for HER2+ breast). Combination use in mCRC is off-label; HERACLES was an Italian Phase 2 study not registrational for a CRC label. · **Guidelines:** Listed in NCCN as 'preferred regimen' for HER2-amplified RAS-WT mCRC alongside tucatinib + trastuzumab. RAS-mutant patients are not on the supportive evidence. · **Geographic scope:** US, EU off-label. · **Verified:** 2026-05-29

Older HER2-CRC combination superseded by tucatinib + trastuzumab. Off-label and RAS-WT-restricted in supporting evidence. Skip in favor of T-DXd for a HER2-amplified A59T patient.

Next steps

- 1. Skip. Route HER2 question to T-DXd if HER2-amplified.

Manufacturer / sponsor contact

- **Company:** Novartis (Tykerb US rights since 2015); Genentech/Roche (Herceptin and biosimilars)
- **Med info phone:**Novartis 1-888-669-6682; Genentech 1-800-821-8590
- **Product info:**<https://www.tykerb.com/>
- **Notes:** Tykerb is no longer manufactured by Novartis in the US since late 2024; supply is being maintained through the existing inventory. Practitioners often default to tucatinib instead.

Payer / coverage: Off-label in mCRC; HERACLES is the supporting evidence but RAS-WT restricted. Same off-label gate as tucatinib + trastuzumab for an A59T patient.

Notes: HERACLES Italian study. Largely superseded by MOUNTAINEER.

30. Trastuzumab deruxtecan (Enhertu, T-DXd)

Aliases: trastuzumab deruxtecan, T-DXd, Enhertu, DS-8201a

Modality: antibody_drug_conjugate · **Regulatory:** FDA-approved April 2024 for HER2-positive (IHC 3+) metastatic solid tumors (tumor-agnostic). The HER2-low and HER2-ultralow breast labels are separate. mCRC use was previously off-label (DESTINY-CRC01/02 evidence) but now sits inside the pan-tumor IHC 3+ label. · **Guidelines:** NCCN category 2A for HER2-amplified mCRC. The label does not restrict by RAS status, so an A59T patient with HER2 IHC 3+ tumor is on-label here. · **Geographic scope:** US, EU, UK, Japan, AU under the pan-tumor IHC 3+ label. · **Verified:** 2026-05-29

The cleanest HER2-pathway option for an A59T patient if HER2 IHC 3+ is confirmed (essential row in target_validation). The 2024 pan-tumor accelerated approval removes the RAS-WT restriction that blocks tucatinib + trastuzumab. ILD monitoring and the deruxtecan payload toxicity profile are the operational considerations.

Next steps

- 1. Order the HER2 IHC + ISH per target_validation; without IHC 3+ (or ISH amp consistent with IHC 3+), this row is moot.
- 2. If HER2-amplified, T-DXd is the labeled HER2-pathway option that does not require RAS-WT.
- 3. Coordinate ILD screening (baseline CT, monthly clinical evaluations) before starting.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04724831		completed	no	—
NCT07427465		recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Daiichi Sankyo / AstraZeneca (co-developed)
- **Med info phone:** Daiichi Sankyo US 1-908-992-6400; AstraZeneca 1-800-236-9933
- **Product info:** <https://www.enhertuhcp.com/>
- **Compassionate / expanded access:** https://www.daiichisankyo.com/our_science/clinical_research/expanded_access/
- **Notes:** Daiichi Sankyo Medical Information at 1-908-992-6400; AstraZeneca medical info at 1-800-236-9933. Patient support via AZ Access 360 (1-844-275-2360).

Payer / coverage: If HER2 IHC 3+, on-label under the 2024 pan-tumor accelerated approval. The patient's KRAS A59T does not disqualify; the gate is HER2 IHC 3+ documentation. Medicare and major commercial payers cover. Interstitial lung disease monitoring is the practical caveat.

Notes: DESTINY-CRC01 and DESTINY-CRC02 anchored the mCRC efficacy. The 2024 tumor-agnostic IHC 3+ label is the operative regulatory path.

31. Tucatinib + trastuzumab (Tukysa + Herceptin, MOUNTAINEER)

Aliases: tucatinib, Tukysa, ONT-380, ARRY-380, trastuzumab, Herceptin, MOUNTAINEER

Modality: small_molecule_plus_mab · **Regulatory:** FDA-approved January 2023 (MOUNTAINEER) for HER2-positive (IHC 3+ or ISH-positive), RAS wild-type metastatic CRC after fluoropyrimidine, oxaliplatin, irinotecan, and an anti-VEGF agent. EMA-approved with comparable wording. · **Guidelines:** NCCN category 2A for HER2-amplified, RAS wild-type mCRC after standard chemo. The label is explicitly RAS-WT; KRAS A59T puts the patient off-label even if HER2-amplified. · **Geographic scope:** US, EU on-label in HER2+ RAS-WT; off-label everywhere else. · **Verified:** 2026-05-29

Approved for HER2-amplified RAS wild-type mCRC; the patient's KRAS A59T puts the labeled indication off the table. If HER2 IHC / ISH (essential row in target_validation) returns amplified, the team faces a hard off-label gate without compendium support. Most realistic call is to skip and route the HER2 question to T-DXd if amplification turns up.

Next steps

1. Wait on the HER2 IHC + ISH result before doing anything (target_validation has this as essential). If HER2 is negative, this row closes.
2. If HER2-amplified despite KRAS A59T, the off-label conversation centers on whether a payer will approve based on the HER2 signal alone. Most decline.
3. T-DXd is the cleaner HER2-amplified path for a RAS-mutant patient — see trastuzumab-deruxtecan row.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05233651		recruiting	no	Pfizer CT.gov Call Center; 1-800-718-1021

NCT03023313	completed	no	—
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Manufacturer / sponsor contact

- **Company:** Seagen / Pfizer (Tukysa); Genentech/Roche (Herceptin and biosimilars)
- **Med info phone:** Pfizer Tukysa: 1-800-438-1985; Genentech 1-800-821-8590
- **Product info:** <https://www.tukysa.com/hcp>
- **Compassionate / expanded access:** <https://www.pfizer.com/science/clinical-trials/expanded-access>
- **Notes:** Pfizer (Tukysa) and Genentech (Herceptin / biosimilars) handle independently.

Payer / coverage: Off-label without RAS wild-type. NCCN Compendium does not currently list tucatinib + trastuzumab for RAS-mutant mCRC. Medical exception is implausible because the BEACON-class precedent (encorafenib + cetuximab benefits only RAS-WT BRAF-V600E) sits next door. The clean call is to skip unless HER2 status is amplified AND a sponsor or trial opens an A59T-eligible cohort.

Notes: MOUNTAINEER is the registrational trial. Label is RAS-WT-restricted.

Clinical trial (15)

32. AZD0466 (dendrimer-conjugated AZD4320 dual Bcl-2 / Bcl-xL)

Aliases: AZD0466, AZD4320 dendrimer

Modality: small_molecule_dendrimer · **Regulatory:** Investigational. Phase 1 first-in-human (NCT04214093 closed; NCT04865419 active in select hematologic indications). · **Guidelines:** Not in NCCN. · **Geographic scope:** US, EU, AU (hematologic trials). · **Verified:** 2026-05-29

Phase 1 dendrimer-formulated dual Bcl-2 / Bcl-xL agent; the dendrimer carrier mitigates thrombocytopenia. No active solid-tumor / mCRC slot today; the program is concentrated in hematologic settings. Route the BCL-xL question through DT2216 (when adult mCRC opens) or stay on the chemo axis.

Next steps

1. Email AZ medical info (1-800-236-9933) to ask whether an mCRC solid-tumor expansion is in active development. Default is to not pursue.

Manufacturer / sponsor contact

- **Company:** AstraZeneca (with Starpharma dendrimer platform)
- **Med info phone:** 1-800-236-9933
- **Product info:** <https://www.astrazeneca.com/>
- **Notes:** AZ medical information handles inquiries. No public solid-tumor mCRC trial at present.

Payer / coverage: Investigational; sponsor-covered if a trial slot exists.

Notes: Dendrimer-conjugated to limit free drug exposure and platelet-cap toxicity.

33. Botensilimab + balstilimab (BOT/BAL, Agenus)

Aliases: botensilimab, AGEN1181, balstilimab, AGEN2034, BOT/BAL

Modality: monoclonal_antibody_combination · **Regulatory:** Investigational. Phase 3 BATTMAN (NCT07152821) is the registrational mCRC trial. Agenus has an FDA-authorized Expanded Access Protocol (NCT06751524) covering multiple cancer types including CRC. France: AAC national access for ovarian and soft-tissue sarcoma. · **Guidelines:** Not in NCCN. Phase 2 data in MSS mCRC (especially without liver metastases) drove the Expanded Access Protocol. · **Geographic scope:** US (USC, MGH, BATTMAN-Canada, EAP nationwide); EU (France AAC for non-CRC indications). · **Verified:** 2026-05-29

The cleanest current MSS-CRC ICI option for this patient. Three operational paths: the USC BOT/BAL + diet + IV vitamin C trial (NCT06336902) which explicitly enrolls KRAS-mutant mCRC, the MGH SBRT trial (NCT07128355) if a peritoneal or hepatic SBRT target can be defined, and the Agenus Expanded Access Protocol (NCT06751524) which is open to treating physicians for chemo-refractory patients. Operational fit improves dramatically once the patient is past 1L FOLFIRI-bev.

Next steps

1. Most actionable today: contact USC's Charlean Ketchens (Charlean.Ketchens@med.usc.edu, 323-865-0451) about NCT06336902 — the KRAS-mutant mCRC cohort is the structural match.
2. Bank the BOT/BAL EAP (NCT06751524) request packet at med.info@agenusbio.com for the moment the patient progresses past 1L; the EAP requires standard-line exhaustion.
3. For SBRT-combination, Aparna Parikh at MGH (aparna.parikh@mgh.harvard.edu) handles NCT07128355 referrals.
4. Keep BATTMAN (NCT07152821) on the radar as the registrational pathway, but it is Canadian-primary.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07152821	3	recruiting	no	Chris O'Callaghan; cocallaghan@ctg.queensu.ca; 613-533-6430
NCT05628044	2	active not recruiting	no	—
NCT06336902	2	recruiting	likely	Charlean Ketchens, RN; Charlean.Ketchens@med.usc.edu; 323-865-0451
NCT07128355	early phase 1	recruiting	unconfirmed	Aparna Parikh, MD; aparna.parikh@mgh.harvard.edu; 617-724-4000
NCT06751524	1	recruiting	unconfirmed	Medical Director; med.info@agenusbio.com; +1-781-202-1614
NCT05627235	2	active not recruiting	no	—
NCT05672216	2	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Agenus
- **Med info phone:** +1-781-202-1614
- **Med info email:** med.info@agenusbio.com
- **Product info:** <https://www.agenusbio.com/>
- **Compassionate / expanded access:** <https://clinicaltrials.gov/study/NCT06751524>
- **Compassionate use email:** med.info@agenusbio.com
- **Notes:** Agenus expanded medical-affairs infrastructure in 2026 specifically to handle increasing physician requests for BOT/BAL EAP access. Treating-physician-initiated requests go to med.info@agenusbio.com.

Payer / coverage: Investigational; sponsor-covered under trial or EAP. EAP coverage of supportive infusion / monitoring varies by institution.

Notes: Botensilimab is the Fc-enhanced anti-CTLA-4 driving the MSS-CRC signal. Peritoneal-only / no-liver-mets cohorts have produced the cleanest responses; the patient's resected-liver + peritoneal profile fits the pattern.

34. Camonsertib (RP-3500, Repare Therapeutics)

Aliases: camonsertib, RP-3500

Modality: small_molecule · **Regulatory:** Investigational. Repare Therapeutics ATR inhibitor; Phase 1/2 TRESR (NCT04497116) basket completed primary analysis; phase 2 expansion in selected biomarker-positive tumors. ·

Guidelines: Not in NCCN. · **Geographic scope:** US + Canada. · **Verified:** 2026-05-29

Active ATR-inhibitor program with Phase 2 expansion in biomarker-positive tumors. The TRESR basket is selective for ATM, BRCA1/2, RNASEH2-deficient tumors rather than broad TP53 mutants, so the patient's TP53 R273 is mechanistically rational but not the dominant TRESR eligibility frame. Route through Repare clinical operations.

Next steps

1. Email clinicaltrials@reparerx.com to ask whether TP53 R273-mutant mCRC fits any active expansion cohort.
2. Confirm whether the comprehensive NGS includes ATM, BRCA, and other HRR genes (which the patient is on a confirmed negative germline panel for).

Manufacturer / sponsor contact

- **Company:** Repare Therapeutics (US development partner Roche for some assets, but RP-3500 retained by Repare)
- **Med info phone:** 857-412-7000
- **Med info email:** info@reparerx.com
- **Product info:** <https://reparerx.com/>
- **Notes:** Repare clinical operations: clinicaltrials@reparerx.com.

Payer / coverage: Investigational; sponsor-covered.

Notes: TRESR phase 1 / 2 platform.

35. Ceralasertib (AZD6738)

Aliases: ceralasertib, AZD6738

Modality: small_molecule · **Regulatory:** Investigational. AstraZeneca ATR inhibitor; multiple combination trials in melanoma, NSCLC, HCC. · **Guidelines:** Not in NCCN. · **Geographic scope:** US, EU, AU. · **Verified:** 2026-05-29

ATR-inhibitor-class agent with mechanistic rationale in TP53-defective (R273) tumors. AZ has multiple active combination trials but no mCRC-specific arm at present. Route through the AZ medical-info line to identify any mCRC-eligible basket.

Next steps

1. Email AZ Medical Information (1-800-236-9933) to ask whether any active ceralasertib basket accepts TP53-mutant mCRC patients.

Manufacturer / sponsor contact

- **Company:** AstraZeneca
- **Med info phone:** 1-800-236-9933
- **Product info:** <https://www.astrazeneca.com/>
- **Compassionate / expanded access:** <https://www.azandmeapp.com/>
- **Notes:** AZ medical info at 1-800-236-9933.

Payer / coverage: Investigational; sponsor-covered under trial.

Notes: Ceralasertib is the most clinically advanced ATR inhibitor.

36. DT2216 (BCL-xL PROTAC degrader)

Aliases: DT2216

Modality: PROTAC_degrader · **Regulatory:** Investigational. FDA Orphan Drug Designation 2022 and Fast Track designation 2023 for T-cell lymphoma. Phase 1 monotherapy in solid tumors completed. No regulatory approval. · **Guidelines:** Not in NCCN. · **Geographic scope:** US trial sites; no adult mCRC arm. · **Verified:** 2026-05-29

Trial-only with no active adult mCRC slot. The paclitaxel-combination trial is restricted to ovarian / endometrial; the COG irinotecan-combination is pediatric. Mechanistically rational for BCL2L1-amplified disease but operationally inaccessible for this adult mCRC patient. The BCL-xL-axis question is open territory; preserve the rationale for an adult mCRC DT2216 cohort if one opens.

Next steps

1. Email info@dialectictx.com to ask whether an adult mCRC arm or a compassionate-use protocol is planned. Document the BCL2L1 amplification status (target_validation amplification-boundary-20q row) when reaching out.
2. Consider AZD0466 as the active alternative dual BCL-2/BCL-xL agent (see AZD0466 row).

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06904009		recruiting	no	Elizabeth Stover, MD, PhD; elizabeth_stover@dfci.harvard.edu; 617-632-6866
NCT06621202		recruiting	no	Michael V Ortiz (MSK PI); 212-639-7592
NCT04886622		completed	no	—

Manufacturer / sponsor contact

- **Company:** Dialectic Therapeutics
- **Med info phone:** 972-499-8190
- **Med info email:** info@dialectictx.com
- **Product info:** <https://www.dialectictx.com/>
- **Notes:** Dialectic is a small biotech; no posted compassionate-use program. Trial-enrollment is the only realistic access path.

Payer / coverage: Investigational; sponsor-covered under trial protocol.

Notes: DT2216 platelet-sparing mechanism vs navitoclax is the design advantage. Phase 1 monotherapy data are encouraging in BCL-xL-dependent disease.

37. Daraxonrasib (RMC-6236)

Aliases: daraxonrasib, RMC-6236, RMC6236

Modality: small_molecule · **Regulatory:** Investigational. FDA Breakthrough Therapy designation for previously treated metastatic KRAS G12X PDAC (2024); FDA Expanded Access Protocol authorized May 2026 for previously treated metastatic PDAC only (not CRC). No FDA, EMA, or PMDA approval for mCRC. · **Guidelines:** Not in NCCN. Trial-enrollment principle applies for atypical KRAS alleles. · **Geographic scope:** US + select EU and Australia sites for RMC-GI-102. · **Verified:** 2026-05-29

Trial-only and gated on whether Revolution Medicines accepts KRAS A59T into the RMC-GI-102 mCRC arms. RASolute-302 phase 3 PDAC is closed and the PDAC Expanded Access Protocol does not cover CRC. Sponsor confirmation on A59T eligibility is the single load-bearing question and must precede any other access action.

Next steps

1. Email medinfo@revmed.com or call 1-844-2-REVMED with the patient's KRAS A59T VAF and clinical summary; ask explicitly whether atypical codon 59 mutations are accepted into the CRC arms of RMC-GI-102 (NCT06445062).
2. If the sponsor accepts A59T, ask which arm fits (monotherapy, +cetuximab — note the patient's atypical-RAS anti-EGFR question — or chemo-combo).
3. If A59T is declined, pivot to JAB-23E73 (NCT06973564) which explicitly enrolls atypical-KRAS alleles.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06447262	1/2	recruiting	unconfirmed	Revolution Medicines; medinfo@revmed.com; 1-844-273-8633
NCT0537285	1/2	recruiting	no	Revolution Medicines; medinfo@revmed.com; 1-844-273-8633

Manufacturer / sponsor contact

- **Company:** Revolution Medicines
- **Med info phone:** 1-844-273-8633
- **Med info email:** medinfo@revmed.com
- **Product info:** <https://www.revolutionmedicines.com/>
- **Compassionate / expanded access:** <https://www.revolutionmedicines.com/patients/expanded-access>
- **Notes:** Revolution Medicines routes medical-info, trial, and expanded-access queries through medinfo@revmed.com / 1-844-2-REVMED. The FDA-authorized Expanded Access Protocol is PDAC-specific; mCRC has no parallel program.

Payer / coverage: Investigational; sponsor covers drug and protocol-mandated assessments. Standard-of-care procedures bill the patient's insurance.

Notes: Daraxonrasib is the most clinically advanced RAS(ON) inhibitor. Sponsor confirmation on A59T is the load-bearing gate.

38. ELI-002 (Elicio 7P KRAS amphiphile peptide vaccine)

Aliases: ELI-002, ELI-002 7P

Modality: vaccine · **Regulatory:** Investigational. Phase 1/2 AMPLIFY-201 / AMPLIFY-7P in MRD-positive resected CRC and PDAC. No regulatory approval. · **Guidelines:** Not in NCCN. · **Geographic scope:** n/a for this patient. · **Verified:** 2026-05-29

Trial-only and enrollment closed. The 7P amphiphile peptide vaccine targets G12C/D/V/R/A/S codon-12 alleles plus G13D — A59T is not in the antigen panel. Closed door on both eligibility and antigen coverage.

Next steps

1. Skip. The KRAS antigen panel does not include A59T.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT0572264	1/2	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Elicio Therapeutics
- **Med info phone:** 857-209-0050
- **Med info email:** medinfo@elicio.com
- **Product info:** <https://www.elicio.com/>
- **Notes:** Elicio is a small biotech; trial-enrollment is the access path.

Payer / coverage: Investigational.

Notes: AMPLIFY-7P closed. A59T is not in the 7P antigen set.

39. Elimusertib (BAY 1895344)

Aliases: elimusertib, BAY 1895344

Modality: small_molecule · **Regulatory:** Investigational. Bayer ATR inhibitor; Phase 1 combination trials. · **Guidelines:** Not in NCCN. · **Geographic scope:** US, EU. · **Verified:** 2026-05-29

Bayer ATR inhibitor without an active mCRC slot at present. Route through Bayer Medical Information to identify any open basket.

Next steps

1. Email Bayer Medical Information (medinfo@bayer.com) to ask about active elimusertib mCRC baskets.

Manufacturer / sponsor contact

- **Company:** Bayer
- **Med info phone:** 1-888-842-2937
- **Med info email:** medinfo@bayer.com
- **Product info:** <https://pharma.bayer.com/>
- **Compassionate / expanded access:** <https://pharma.bayer.com/expanded-access-and-compassionate-use>

Payer / coverage: Investigational.

Notes: Elimusertib is the Bayer ATR inhibitor.

40. GRT-C901 + GRT-R902 (Gritstone personalized neoantigen vaccine)

Aliases: GRT-C901, GRT-R902, Gritstone GO-010

Modality: vaccine · **Regulatory:** Investigational. Phase 2/3 GO-010 in MSS mCRC. · **Guidelines:** Not in NCCN. · **Geographic scope:** n/a · **Verified:** 2026-05-29

Closed door. Gritstone bankruptcy and trial closure mean no current patient access. Listed so the team does not chase the program.

Next steps

- 1. Skip. Route the personalized-vaccine question to KRAS peptide + BOT/BAL (Johns Hopkins NCT06411691) if vaccines remain in interest.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05121321	2/3	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Seattle Project Corporation (acquired Gritstone bio assets late 2024 after Gritstone bankruptcy)
- **Notes:** Gritstone bio filed Chapter 11 October 2024. Seattle Project Corporation acquired the neoantigen vaccine assets and is maintaining the trial through follow-up but not enrolling. No active patient-access pathway.

Payer / coverage: Investigational; trial enrollment closed.

Notes: Gritstone bankruptcy October 2024. Seattle Project Corporation maintaining the protocol for follow-up only.

41. JAB-23E73 (Jacobio pan-KRAS inhibitor)

Aliases: JAB-23E73, JAB23E73

Modality: small_molecule · **Regulatory:** Investigational. Phase 1/2 first-in-human. No regulatory approval. · **Guidelines:** Not in NCCN. · **Geographic scope:** United States — 4 sites. · **Verified:** 2026-05-29

Trial-only with four US sites. JAB-23E73 explicitly studies atypical KRAS alleles, which makes it the highest-priority US fallback if Revolution Medicines does not accept A59T into RMC-GI-102. Mayo Rochester is the highest-volume site for the patient's profile.

Next steps

- 1. Email clinicaltrials@jacobiopharma.com or call 781-918-6670 with the patient's KRAS A59T VAF and ask which of the four sites has open screening slots.
- 2. Confirm whether the patient's planned CRS-HIPEC window can be accommodated (screen-and-bank vs immediate enrollment).
- 3. Mayo Rochester is the operational default given trial volume and surgical-medical-oncology coordination.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06972564	1/2	recruiting	likely	Jacobio Pharmaceuticals; clinicaltrials@jacobiopharma.com ; 781-918-6670

Manufacturer / sponsor contact

- **Company:** Jacobio Pharmaceuticals
- **Med info phone:** 781-918-6670
- **Med info email:** clinicaltrials@jacobiopharma.com
- **Product info:** <https://www.jacobiopharma.com/en/pipeline>
- **Notes:** Jacobio's US-based clinical operations team handles inquiries at the Boston number. No public compassionate-use program.

Payer / coverage: Investigational; sponsor-covered drug and protocol procedures.

Notes: Phase 1/2 pan-KRAS inhibitor with explicit atypical-KRAS scope. The structural match for KRAS A59T.

42. KRAS peptide vaccine + Poly-ICLC + balstilimab + botensilimab (Johns Hopkins)

Aliases: mKRAS vaccine, Johns Hopkins KRAS vaccine + BOT/BAL

Modality: vaccine_plus_ici · **Regulatory:** Investigational. · **Guidelines:** Not in NCCN. · **Geographic scope:** United States (Johns Hopkins). · **Verified:** 2026-05-29

Active Phase 1 academic trial pairing a KRAS peptide vaccine with the BOT/BAL ICI combination. The vaccine antigen set is the load-bearing question — most synthetic long peptide designs target the canonical G12 hotspots, and A59T may or may not be in scope. Sponsor confirmation is the gate.

Next steps

1. Email GIClinicalTrials@jhmi.edu or call Colleen Apostol RN (410-614-3644) with the patient's KRAS A59T molecular profile to ask whether the vaccine antigen set includes codon 59.
2. If A59T is excluded, this row closes. If included, the trial is the cleanest current KRAS-vaccine + ICI option.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06411691		recruiting	unconfirmed	Colleen Apostol, RN; GIClinicalTrials@jhmi.edu ; 410-614-3644

Manufacturer / sponsor contact

- **Company:** Sidney Kimmel Comprehensive Cancer Center at Johns Hopkins (academic IND); Agenus supplies BOT/BAL
- **Med info phone:** 410-614-3644
- **Med info email:** GIClinicalTrials@jhmi.edu
- **Product info:** <https://www.hopkinsmedicine.org/sidney-kimmel-comprehensive-cancer-center/>
- **Notes:** Academic IND program. Agenus provides BOT/BAL under sponsor agreement.

Payer / coverage: Investigational; sponsor-covered under trial.

Notes: Academic Johns Hopkins IND with Agenus BOT/BAL combination. A59T antigen inclusion is the load-bearing gate.

43. Pelcitoclax (APG-1252)

Aliases: pelcitoclax, APG-1252

Modality: small_molecule · **Regulatory:** Investigational. Phase 1 / 2 trials ongoing, mostly Asian sites; some US clinical-trial activity. · **Guidelines:** Not in NCCN. · **Geographic scope:** US + China + Asia-Pacific. · **Verified:** 2026-05-29

Phase 1 dual Bcl-2 / Bcl-xL inhibitor with active combination trials in SCLC and lymphoma but no adult mCRC slot today. Mechanistically interchangeable with DT2216 for BCL-xL coverage; preserve the rationale and watch for an mCRC expansion.

Next steps

1. Email info@ascentage.com to ask about adult mCRC expansion plans; default is not to pursue today.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03080311		completed	no	—

Manufacturer / sponsor contact

- **Company:** Ascentage Pharma
- **Med info phone:** 240-552-6051
- **Med info email:** info@ascentage.com
- **Product info:** <https://www.ascentagepharma.com/>
- **Notes:** Ascentage operates US trial sites for several programs; clinical operations contact via clinicaltrials@ascentage.com.

Payer / coverage: Investigational.

Notes: Dual Bcl-2 / Bcl-xL inhibitor in the same mechanistic class as navitoclax and DT2216.

44. Pressurized intraperitoneal aerosol chemotherapy (PIPAC)

Aliases: PIPAC, PIPAC-oxaliplatin, PIPAC-mitomycin

Modality: surgical_procedure · **Regulatory:** Investigational in the US. PIPAC aerosolization equipment (Capnopen) is CE-marked in EU; not FDA-cleared as a system, so PIPAC is delivered under research protocols in the US. · **Guidelines:** Not yet in NCCN guidelines. ESMO acknowledges as a developing modality. · **Geographic scope:** United States (3 trial sites) — investigational only. · **Verified:** 2026-05-29

Trial-only in the US, with three centers (City of Hope, Mayo Jacksonville, Northwell) on NCT04329494. Operationally relevant only if the team defers the planned CRS-HIPEC; otherwise PIPAC sits as the backup for peritoneal recurrence after CRS-HIPEC fails.

Next steps

1. Reserve PIPAC for post-HIPEC peritoneal recurrence; not in scope while the upfront CRS-HIPEC is the plan.
2. If the team pivots to PIPAC instead of CRS-HIPEC, contact Thanh Dellinger at City of Hope (tdellinger@coh.org) — the most established US site.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04329494		recruiting	likely	Thanh H. Dellinger, MD; tdellinger@coh.org; 626-218-1379
NCT06327270		recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Capnomed (Capnopen device); chemo drugs are generic
- **Notes:** Capnopen aerosolization device manufactured by Capnomed (Villingendorf, Germany). PIPAC chemo agents are oxaliplatin or mitomycin C generic.

Payer / coverage: PIPAC is investigational in the US — coverage runs through the trial, not insurance. Outside a trial, PIPAC is not currently available in the US.

Notes: CRC-PIPAC and the PRECINCT registry support feasibility; survival evidence is registry-level. Sugarbaker network is rolling out PIPAC at additional US centers.

45. S241656 (Servier ERK inhibitor, BDTX-4933-101)

Aliases: S241656, BDTX-4933

Modality: small_molecule · **Regulatory:** Investigational. Phase 1/2 first-in-human. No regulatory approval. ·

Guidelines: Not in NCCN. · **Geographic scope:** US + EU. · **Verified:** 2026-05-29

ERK-inhibitor trial with multiple US sites and explicit MAPK-pathway eligibility. Mechanistically rational for KRAS A59T (downstream blockade) but the sponsor needs to confirm A59T qualifies. Cleaner US access than daraxonrasib if A59T is excluded from RMC-GI-102.

Next steps

1. Email scientificinformation@servier.com with the patient's KRAS A59T molecular profile to ask whether codon 59 alterations qualify under the MAPK-pathway eligibility.
2. If accepted, MSK, Dana-Farber, and Washington University are the highest-volume US sites.
3. Combination arm selection (mono vs +chemo vs +anti-EGFR) interacts with the anti-EGFR-resistance question for A59T — confirm at screening.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
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NCT05781624

recruiting

likely

Servier I.R.I.S. Clinical Studies
Department;
scientificinformation@servier.com;
+33 1 55 72 60 00

Manufacturer / sponsor contact

- **Company:** Servier (Institut de Recherches Internationales Servier)
- **Med info phone:** 1-866-563-7842
- **Med info email:** scientificinformation@servier.com
- **Product info:** <https://www.servier.com/>
- **Notes:** Servier Pharmaceuticals US operates in Boston, MA. US medical information: 1-866-7-Servier.

Payer / coverage: Investigational; sponsor-covered under protocol.

Notes: ERK inhibition as downstream MAPK blockade for atypical KRAS. The combination arms include anti-EGFR partners which is unusual for an A59T patient.

46. ST316 (Sapience Therapeutics beta-catenin / BCL9 disruptor)

Aliases: ST316

Modality: peptide_antagonist · **Regulatory:** Investigational. Phase 1/2 first-in-human. · **Guidelines:** Not in NCCN. · **Geographic scope:** United States. · **Verified:** 2026-05-29

Trial-only with active US enrollment. APC E1295 (MCR-region truncation) is the structural target the trial is built around. The CRC expansion cohort is the operational entry. Sponsor confirmation on the exact E1295 variant call (target_validation row apc-e1295-mcr-position) is the gating prerequisite.

Next steps

1. Email jgakuria@sapiencetherapeutics.com or call 914-418-5100 with the patient's APC E1295 variant call and ask whether MCR-region truncating mutations are accepted into the CRC expansion cohort.
2. Resolve the E1295 variant type (target_validation apc-e1295-mcr-position row) before the screening call.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05848739		recruiting	likely	Joyce Gakuria; jgakuria@sapiencetherapeutics.com ; 914-418-5100

Manufacturer / sponsor contact

- **Company:** Sapience Therapeutics
- **Med info phone:** 914-418-5100
- **Med info email:** info@sapiencetherapeutics.com
- **Product info:** <https://www.sapiencetherapeutics.com/>

- **Notes:** Sapience is a small biotech; clinical operations contact is the trial central contact.

Payer / coverage: Investigational; sponsor-covered under protocol.

Notes: Sapience peptide antagonist disrupting the beta-catenin / BCL9 / STAT3 interaction. APC-truncated CRC is the structurally aligned indication.

Not yet accessible (1)

47. JYP0015 (Guangzhou JOYO pan-RAS inhibitor)

Aliases: JYP0015

Modality: small_molecule · **Regulatory:** Investigational. Phase 1/2 in China only. · **Guidelines:** Not in NCCN.
 · **Geographic scope:** Mainland China only. · **Verified:** 2026-05-29

Mainland-China-only trial. Geographically out of reach for the US-based patient. Note as a parallel program for situational awareness; no US access path.

Next steps

1. Do not pursue. Route the pan-RAS question through daraxonrasib (RMC-GI-102) or JAB-23E73.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06894231	1/2	recruiting	no	Ling Shen, MD; linshenpku@163.com; +86-010-88121122

Manufacturer / sponsor contact

- **Company:** Guangzhou JOYO Pharma
- **Notes:** No US presence; no medical-information line for US clinicians.

Payer / coverage: Not applicable for US patients.

Notes: JYP0015 is structurally analogous to daraxonrasib but the program is China-domestic.

Unavailable (14)

48. Adagrasib (Krazati) ± cetuximab (KRYSTAL-1, KRAS G12C mCRC)

Aliases: adagrasib, MRTX849, Krazati, KRYSTAL-1

Modality: small_molecule_plus_mab · **Regulatory:** Adagrasib FDA-approved December 2022 (KRAS G12C NSCLC); accelerated approval June 2024 (KRYSTAL-1) for adagrasib + cetuximab in KRAS G12C mCRC. Allele-specific. · **Guidelines:** NCCN category 2A for KRAS G12C mCRC only. · **Geographic scope:** n/a · **Verified:** 2026-05-29

Closed door for A59T. Krazati is G12C-specific. Listed for completeness.

Next steps

1. Skip.

Manufacturer / sponsor contact

- **Company:** Bristol Myers Squibb (acquired Mirati 2024)
- **Med info phone:** 1-800-721-5072
- **Product info:** <https://www.krazati.com/>
- **Notes:** BMS Medical Information operates 1-800-721-5072 for US HCP inquiries.

Payer / coverage: Not applicable to A59T.

Notes: Allele-specific G12C-binder.

49. Adagrasib + pembrolizumab (KRYSTAL-7, KRAS G12C NSCLC)

Aliases: adagrasib, pembrolizumab, KRYSTAL-7

Modality: small_molecule_plus_mab · **Regulatory:** Investigational combination. Allele-specific G12C target. · **Guidelines:** Not applicable to A59T. · **Geographic scope:** n/a · **Verified:** 2026-05-29

Closed door for A59T. NSCLC-G12C indication.

Next steps

1. Skip.

Manufacturer / sponsor contact

- **Company:** Bristol Myers Squibb + Merck
- **Med info phone:** BMS 1-800-721-5072; Merck 1-800-672-6372
- **Notes:** Same BMS contacts as adagrasib.

Payer / coverage: Not applicable to A59T.

Notes: Listed for completeness so the team does not chase a non-applicable G12C combination.

50. Atezolizumab + cobimetinib (IMblaze370)

Aliases: atezolizumab, Tecentriq, cobimetinib, Cotellic, IMblaze370

Modality: monoclonal_antibody_plus_smi · **Regulatory:** Each drug FDA-approved separately. The atezolizumab + cobimetinib mCRC combination was not approved; IMblaze370 failed primary endpoint vs regorafenib in 3L MSS-CRC. · **Guidelines:** Not in NCCN. · **Geographic scope:** n/a · **Verified:** 2026-05-29

Closed door. IMblaze370 was negative and Genentech did not pursue the indication. Other ICI combinations are the more productive lanes (BOT/BAL).

Next steps

1. Skip.

Manufacturer / sponsor contact

- **Company:** Genentech/Roche
- **Med info phone:** 1-800-821-8590
- **Med info email:** med.info@gene.com
- **Product info:** <https://www.gene.com/medical-professionals/medicines/tecentriq>

Payer / coverage: Not applicable — combination not labeled for mCRC and the supportive trial failed.

Notes: IMblaze370 negative phase 3.

51. Atypical KRAS / NRAS variant functional characterization

Aliases: atypical KRAS A59 codon

Modality: biology · **Regulatory:** n/a — biology / characterization row. · **Guidelines:** n/a · **Geographic scope:** n/a · **Verified:** 2026-05-29

Functional-characterization entry, not a drug. The A59T allele's switch-II region mechanics inform pan-RAS / RAS(ON) selection. Drug-access pathway routes through daraxonrasib, JAB-23E73, S241656 — see those rows.

Next steps

1. Route to the pan-RAS / RAS(ON) drug rows for action.

Payer / coverage: n/a

Notes: Cross-references the daraxonrasib, JAB-23E73, S241656 access rows.

52. Encorafenib + cetuximab (Braftovi + Erbitux, BEACON)

Aliases: encorafenib, Braftovi, LGX818, cetuximab, Erbitux, BEACON

Modality: small_molecule_plus_mab · **Regulatory:** FDA-approved April 2020 (BEACON CRC) for BRAF V600E-mutated mCRC after one prior line of therapy. FDA-approved December 2024 (BREAKWATER) for 1L BRAF V600E mCRC. Indication is BRAF V600E-specific. · **Guidelines:** NCCN category 1 for BRAF

V600E-mutated mCRC. Only applicable if BRAF V600E co-occurs; profile.json flags BRAF status as not explicitly reported. · **Geographic scope:** n/a unless BRAF V600E positive. · **Verified:** 2026-05-29

Closed door because (a) BRAF V600E is not reported in the profile and (b) the cetuximab component is off-label in RAS-mutant tumors. Most likely BRAF V600E is wild-type once confirmed; if it is V600E, the team still faces the off-label cetuximab gate.

Next steps

1. Confirm BRAF V600E status from the comprehensive NGS (target_validation essential row). If V600E is positive AND the team wants to consider this regimen despite the A59T anti-EGFR off-label question, route through the sponsor for guidance; otherwise skip.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04637421		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Pfizer (Array BioPharma acquisition); Lilly (cetuximab)
- **Med info phone:** Pfizer 1-800-438-1985; Lilly 1-800-545-5979
- **Product info:** <https://www.braftovi.com/hcp>
- **Notes:** Same Pfizer contacts as other Pfizer oncology assets.

Payer / coverage: Not applicable unless BRAF V600E is positive. Confirmed RAS-mutant tumors fail anti-EGFR labeling, so even a BRAF V600E + KRAS A59T tumor would be off-label for cetuximab.

Notes: BEACON CRC and BREAKWATER are the registrational trials. RAS-mutant + BRAF V600E is unusual and not well-studied.

53. Eprenetapopt (APR-246, PRIMA-1-MET)

Aliases: eprenetapopt, APR-246, PRIMA-1-MET

Modality: small_molecule · **Regulatory:** Investigational; development program at Aprea Therapeutics was substantially de-emphasized after the phase 3 MDS trial (NCT03745716) failed in 2020. Aprea pivoted toward ATR / WEE1 inhibitors. No active enrollment for TP53 R273 solid-tumor cohorts. · **Guidelines:** Not in NCCN. · **Geographic scope:** n/a · **Verified:** 2026-05-29

Effectively discontinued for solid tumors. The TP53 R273 reactivation program is closed. Listed so the team does not pursue a phantom path.

Next steps

1. Treat as a closed door for this patient. The TP53 R273 question moves to synthetic-lethal ATR-class agents (see ATR row).

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04381238		completed	no	—

Manufacturer / sponsor contact

- **Company:** Aprea Therapeutics
- **Med info phone:** 1-617-463-9385
- **Med info email:** info@aprea.com
- **Product info:** <https://www.aprea.com/>
- **Notes:** Aprea has shifted clinical focus to ATR / WEE1 inhibitors (ATRN-119, ATRN-1051). Eprenetapopt is not in active enrollment.

Payer / coverage: Not applicable.

Notes: Aprea's TP53 program effectively wound down. The R273H structural-reactivation pharmacology remains an academic open question.

54. Galunisertib (TGFbR1 inhibitor) + nivolumab

Aliases: galunisertib, LY2157299, nivolumab

Modality: small_molecule_plus_mab · **Regulatory:** Galunisertib clinical program discontinued by Lilly after the HCC phase 2 readout. No active enrollment in mCRC. · **Guidelines:** Not in NCCN. · **Geographic scope:** n/a · **Verified:** 2026-05-29

Closed door. Galunisertib is no longer in active development. The TGF-beta-axis question for the patient's SMAD4 R361H biology routes more cleanly through bintrafusp alfa-class agents, NIS793 (Novartis), or BCA101 — none of which currently have an mCRC slot. Net: not actionable today.

Next steps

1. Skip. The TGF-beta-axis question is not actionable today; preserve the rationale for future trial opportunities.

Manufacturer / sponsor contact

- **Company:** Eli Lilly (galunisertib); BMS (nivolumab)
- **Med info phone:** Lilly 1-800-545-5979; BMS 1-800-721-5072
- **Notes:** Lilly discontinued galunisertib clinical development.

Payer / coverage: Not applicable.

Notes: Galunisertib was the most clinically advanced TGFbR1 inhibitor and Lilly closed the program.

55. Navitoclax (ABT-263)

Aliases: navitoclax, ABT-263

Modality: small_molecule · **Regulatory:** Investigational; no FDA approval. AbbVie discontinued solid-tumor development after thrombocytopenia limited clinical utility. Myelofibrosis program (navitoclax + ruxolitinib, TRANSFORM-1) failed primary endpoint 2024. · **Guidelines:** Not in NCCN. · **Geographic scope:** n/a · **Verified:** 2026-05-29

Closed door. AbbVie discontinued the solid-tumor program because of dose-limiting thrombocytopenia and the TRANSFORM-1 myelofibrosis failure narrowed remaining indications. Route BCL-xL-axis access through DT2216 (when an adult mCRC arm opens) or AZD0466.

Next steps

1. Skip. The BCL-xL-axis question routes through DT2216 or AZD0466.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT01009073		completed	no	—

Manufacturer / sponsor contact

- **Company:** AbbVie
- **Med info phone:** 1-800-633-9110
- **Product info:** <https://www.abbvie.com/>
- **Notes:** AbbVie Medical Information at 1-800-633-9110. Navitoclax solid-tumor program effectively closed.

Payer / coverage: Not applicable.

Notes: Foundational BH3-mimetic; mechanism preserved by the next-generation platelet-sparing agents.

56. RMC-7977 (Revolution Medicines RAS(ON) tool compound)

Aliases: RMC-7977

Modality: small_molecule · **Regulatory:** Preclinical tool compound. Not in clinical development. · **Guidelines:** n/a · **Geographic scope:** Not applicable. · **Verified:** 2026-05-29

Preclinical only. Not a patient-accessible drug; the clinical program is daraxonrasib (RMC-6236).

Next steps

1. Treat RMC-7977 as preclinical evidence supporting daraxonrasib mechanism; route patient access through the daraxonrasib (RMC-6236) row.

Manufacturer / sponsor contact

- **Company:** Revolution Medicines
- **Med info phone:** 1-844-273-8633
- **Med info email:** medinfo@revmed.com
- **Notes:** RMC-7977 is the preclinical analog used to map RAS(ON) pharmacology; daraxonrasib (RMC-6236) is the clinical successor. No patient-accessible pathway.

Payer / coverage: Not applicable.

Notes: Preclinical tool compound for the RAS(ON) class.

57. Rezatapopt (PC14586, PYNNACLE)

Aliases: rezatapopt, PC14586

Modality: small_molecule · **Regulatory:** Investigational. PMV Pharmaceuticals Phase 1/2 PYNNACLE (NCT04585750) Y220C-restricted; FDA Fast Track designation for TP53 Y220C solid tumors. · **Guidelines:** Not in NCCN. · **Geographic scope:** n/a · **Verified:** 2026-05-29

Closed door for this patient. Rezatapopt is Y220C-specific (the drug binds a Y220C-specific surface pocket) and PYNNACLE additionally requires KRAS wild-type. The patient is TP53 R273 and KRAS A59T-mutant — double-exclusion. Listed only to document the rule-out so the trial-screening conversation does not loop back.

Next steps

1. Skip. Confirm via target_validation tp53-y220c-ruleout row that there is no co-occurring Y220C variant; if confirmed, this row closes formally.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04585750		recruiting	no	PMV Pharma Clinical Study Information Center; clinicaltrials@pmvpharma.com ; 609-235-4038

Manufacturer / sponsor contact

- **Company:** PMV Pharmaceuticals
- **Med info phone:** 609-235-4038
- **Med info email:** clinicaltrials@pmvpharma.com
- **Product info:** <https://www.pmvpharma.com/>
- **Notes:** PMV Pharma operates from Cranbury, NJ. No compassionate-access program for non-Y220C variants.

Payer / coverage: Not applicable.

Notes: PYNNACLE pivotal trial. Y220C is the binding-pocket-specific allele.

58. SMAD4 R361H biology — prognostic / no targeted therapy

Aliases: SMAD4 prognostic biology, CMS4 mesenchymal

Modality: biomarker · **Regulatory:** n/a — biomarker / biology row. · **Guidelines:** n/a · **Geographic scope:** n/a · **Verified:** 2026-05-29

Biomarker / biology entry, not a drug. SMAD4 R361H supports CMS4-mesenchymal phenotype with TGF-beta-mediated immune exclusion. No directly targetable agent today; informs ICI-combination trial selection (BOT/BAL + SBRT, or BOT/BAL + AGEN1423) and CMS classification (target_validation cms-class-cdx2-mesenchymal row).

Next steps

1. Resolve CMS class from the transcriptomic or IHC-surrogate panel (target_validation cms-class-cdx2-mesenchymal row). If CMS4, the TGF-beta-axis story becomes load-bearing for combination ICI trial selection.

Payer / coverage: n/a

Notes: SMAD4 R361H is prognostic, not yet directly druggable. Route the biology through CMS classification and ICI combination selection.

59. Sotorasib + ICI combinations (CodeBreakK 101)

Aliases: sotorasib, AMG 510, CodeBreakK 101

Modality: small_molecule_plus_mab · **Regulatory:** Sotorasib + ICI combinations are investigational (CodeBreakK 101 platform). Sotorasib monotherapy is FDA-approved for KRAS G12C NSCLC and CRC; ICI combinations are exploratory. A59T not in scope. · **Guidelines:** Not applicable to A59T. · **Geographic scope:** n/a · **Verified:** 2026-05-29

Closed door for A59T. Sotorasib targets G12C only. Listed for completeness so the team does not chase a phantom option.

Next steps

1. Skip.

Manufacturer / sponsor contact

- **Company:** Amgen
- **Med info phone:** 1-800-772-6436
- **Med info email:** medinfo@amgen.com
- **Notes:** Same Amgen contacts as panitumumab.

Payer / coverage: Not applicable.

Notes: G12C-allele-specific. Hepatotoxicity in sotorasib + pembrolizumab limits the combination separately.

60. Sotorasib + panitumumab (CodeBreak 300, KRAS G12C mCRC)

Aliases: sotorasib, Lumakras, AMG 510, sotorasib + panitumumab, CodeBreak 300

Modality: small_molecule_plus_mab · **Regulatory:** Sotorasib + panitumumab FDA-approved January 2025 (CodeBreak 300) for KRAS G12C-mutated mCRC after fluoropyrimidine, oxaliplatin, and irinotecan. Indication is specifically G12C; A59T is not in scope. · **Guidelines:** NCCN category 1 for KRAS G12C mCRC only. Not applicable to KRAS A59T. · **Geographic scope:** n/a · **Verified:** 2026-05-29

Closed door. Sotorasib binds the GDP-bound G12C switch-II pocket; the A59 substitution does not generate the same drug-trapping pocket. Do not pursue.

Next steps

1. Skip. Route the KRAS question through pan-RAS / RAS(ON) inhibitors (daraxonrasib, JAB-23E73) instead.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07124884		not yet recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Amgen
- **Med info phone:** 1-800-772-6436
- **Med info email:** medinfo@amgen.com
- **Product info:** <https://www.lumakrashcp.com/>
- **Notes:** Same Amgen contacts as panitumumab.

Payer / coverage: Not applicable — drug is allele-specific for G12C and the patient is A59T.

Notes: Mechanistically allele-specific. Explicitly flagged as a closed door for A59T patients.

61. WNT974 (LGK974) porcupine inhibitor

Aliases: WNT974, LGK974

Modality: small_molecule · **Regulatory:** Investigational. Novartis Phase 1 (NCT01351103) completed. The program substantially de-prioritized after dose-limiting bone toxicity; no current active solid-tumor enrollment. · **Guidelines:** Not in NCCN. · **Geographic scope:** n/a · **Verified:** 2026-05-29

Effectively dormant. Porcupine inhibitors face an on-target bone-toxicity dose-limit that has stalled the class. Route APC E1295 access through ST316 instead.

Next steps

1. Skip. ST316 is the active Wnt-axis lane.

Manufacturer / sponsor contact

- **Company:** Novartis
- **Med info phone:** 1-888-669-6682
- **Med info email:** medinfo.us@novartis.com
- **Product info:** <https://www.novartis.com/>
- **Notes:** Novartis Managed Access Program at compassionate.use@novartis.com handles pre-approval access but WNT974 is not in active enrollment.

Payer / coverage: Not applicable.

Notes: Porcupine inhibition is a mechanism-class-limited approach due to bone toxicity.
